ClearPoint Neuro (Nasdaq: CLPT)

Company Update: FY2026E–FY2032E Revenue and EBITDA Forecast

October 25, 2026. US$ millions unless stated. Scenario forecast: it assumes uniQure's AMT-130 and Bayer/BlueRock's bemdaneprocel are approved, counts every one-time Parkinson's cell/gene therapy procedure as one class delivered on ClearPoint's platform, and prices every procedure at $20k, as requested. AMT-130 volume follows ClearPoint's stated capacity; Parkinson's volume ramps over three years to modelled class demand. It includes a Finagle discounted-cash-flow valuation but carries no rating. Every tag resolves in Appendix B. Appendix C sets out the Parkinson's eligible-pool model behind the Parkinson's class line, and Appendix D lists ClearPoint's partner programs; outside the Parkinson's class and AMT-130 they are not in the forecast.

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1. Summary

FORECAST UPDATE

■ **We forecast revenue rising from $49.0M in FY2026E to $320M in FY2030E and $429M in FY2032E [E1][C7].** FY2026E sits on analyst consensus after the August guidance cut to $48.0–52.0M [G2][E1]. The core business follows consensus to $56.6M in FY2027E, then grows 15% a year, the low end of the 15–20% Burnett talks about [E1][G12][S7][S26] ★. Procedure revenue at $20k each [P2] adds $6.6M in FY2027E, $282M in FY2031E and $279M in FY2032E [C7]. The CAL preclinical laboratory adds $7.0M in FY2028E, rising to $36.0M in FY2032E, 60% of the $60M+ capacity management cites [G9][S37] ★. Growth peaks at 93% in FY2028E as AMT-130 launches and stays high at 89% in FY2029E as the Parkinson's class launches. It falls to 4.4% in FY2032E, when Parkinson's demand eases after the launch backlog is drawn down, then runs at about 5% through FY2045E under Finagle's fade [C29][C34].

■ **EBITDA turns positive in FY2028E at $12.0M and reaches $177.5M (a 41% margin) in FY2032E [S15][C10].** Procedure revenue carries an assumed 70% gross margin [S23], 10% of revenue for clinical specialists and $2M a year per therapy [S24][S25]. CAL revenue earns the core gross margin less 20% incremental opex [G9][S38] ★. FY2026E and FY2027E stay loss-making at −$23.6M and −$19.5M [C9][S15].

■ **One-time Parkinson's cell/gene therapy procedures (the class, all on ClearPoint's platform) ramp over three years from the FY2029E launch to class demand: about 3,900 in FY2029E, 7,600 in FY2030E and 11,100 in FY2031E, then 10,900 in FY2032E and about 11,500 in FY2040E [S31][S62][S63][S64][C40] ★.** We size demand for the whole class of one-time Parkinson's cell and gene therapies from published prevalence, label-style eligibility (levodopa-responsive, at least four years since diagnosis, motor complications, no dementia, fit for surgery) and a stock-and-flow pool (Appendix C) [A30][C42]. About 180,000 US patients are medically eligible in FY2026E, 16% of PD [C42]. DBS and one-time therapy are not treated as mutually exclusive: trials exclude patients who have had DBS, but that is an entry rule and we found no biological or safety reason for it, so these patients stay in the pool [S44][A41]–[A43] ★. About 3,990 eligible patients receive DBS each year, 2.2% of the pool [A10][C49]. We assume 6% of the pool a year would receive a one-time, potentially disease-modifying therapy from FY2029E, about 2.7 times the DBS rate. DBS continues alongside and is not deducted [S45] ★. Class demand is about 11,800 in the launch year, eases to about 10,800 in FY2033E–35E as the backlog is drawn down, then rises with the pool to 11,500 in FY2040E [C40]. Volume reaches demand in FY2031E. The three-year ramp is David's judgment that it also gives time for other Parkinson's therapies to be approved [S64] ★. Running 11,100 Parkinson's procedures a year needs about 74 centres at 3 cases a week, far beyond the 20–30 centres management talks about, an aggressive judgment [S43][G7] ★. We treat all one-time PD cell/gene procedures as one class on ClearPoint's platform (100% share, David's view) [S62][S63] ★. The bull treats 8% of the pool a year on the same ramp, giving 14,300 procedures in FY2031E [S59][C46] ★. The bear excludes patients who have had DBS, as trials do (pool about 138,000), and stays limited by its 4,500 capacity after a 2030 approval [S22][S43][C55]. AMT-130 is unchanged at 3,000 a year from FY2030E [S30].

■ **Finagle values the shares at $54.29 in the base case vs. a $9.25 price (derived) [C14][C17].** The bull case gives $72.68 and the bear $16.72 [C15][C16]. The value comes mainly from the Parkinson's volume path. If Parkinson's volume were held at a 5,000 capacity, the 180,000 pool would change nothing (base $39.25), because capacity rather than demand

would bind; ramping to class demand of about 11,000 a year adds about $15 (Figure 8c) [C48]. Volume reaches class demand in FY2031E, so there is no step after FY2032E for Finagle to compound; FY2033E growth (gsnext) is 5.1% [C29]. About 49% of base value is terminal value (derived) [C27]. Each additional $20M of annual procedure revenue from FY2030E (1,000 patients at $20k) adds about $2.69 a share in Finagle [C43]. Cash shortfalls are funded as negative dividends, not dilution. These are scenario values, not a target.

Figure 1 shows the procedure line turning losses into profits from FY2028E.

Figure 1 – Financial summary (base case)

US$M	FY2024A	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E
Revenue	31.4	37.0	49.0	63.1	122.0	230.6	319.9	410.4	428.6
Revenue growth	–	17.8%	32.5%	28.9%	93.2%	88.9%	38.7%	28.3%	4.4%
Gross margin	–	61.4%	62.9%	63.6%	65.8%	67.3%	67.6%	67.8%	67.5%
EBITDA (before SBC)	–	−15.2	−23.6	−19.5	12.0	71.2	120.6	170.8	177.5
EBITDA growth	–	–	n/m	n/m	n/m	493.6%	69.4%	41.6%	3.9%
EBITDA margin	–	−41.0%	−48.1%	−30.9%	9.8%	30.9%	37.7%	41.6%	41.4%

Source: A1, A3, C7, C8, C9, C10, C11, E1, S13, S15, S23, S32, S37, S38. EBITDA growth is not meaningful (n/m) on a negative base. FY2024A EBITDA is not built in this forecast.

2. Business Description and Reporting Lines

ClearPoint Neuro sells an MRI- and CT-guided platform for delivering therapies into the brain: frames, cannulae, software and services. Its customers are hospitals (for neurosurgery) and biopharma companies (for cell and gene therapy trials) [A2]. It reports three lines [A2]:

- **Biologics and drug delivery (51% of FY2025A revenue).** Disposables, clinical case support, preclinical services, now run from the CAL (ClearPoint Advanced Laboratories), a 30,000 sq ft preclinical contract-research facility in Torrey Pines, San Diego [D2][G9], and licensing/milestone fees from more than 60 pharma partners, including uniQure and BlueRock [A2][G2]. PTC Therapeutics accounted for 15% of this line in FY2025A [D1]. Revenue is earned per procedure and per service contract. Once a partner product is approved, ClearPoint expects about $12–15k per patient across partners [G5] and $15–25k per typical uniQure procedure [G4]. This forecast prices both approved therapies at $20k per procedure [P2].
- **Neurosurgery navigation and therapy (40%).** Disposables for deep-brain stimulation, laser ablation and biopsy, plus the IRRAflow drainage system added with the IRRAS acquisition in November 2025 [A2][D1]. These are sold directly to hospitals, mostly in the US. The five largest hospitals make up 21% of disposables [D1].
- **Capital equipment and software (8%).** ClearPoint consoles, robotics, the operating-room navigation software and the IRRAflow control units, sold as capital or placed [A2].

Figure 2 shows that biologics and drug delivery is still the largest line, but neurosurgery is gaining share quickly.

Figure 2 – Revenue by line, FY2025A

Line	US$M	Share	YoY
Biologics and drug delivery	19.0	51.5%	10.0%
Neurosurgery navigation and therapy	14.8	40.1%	44.2%
Capital equipment and software	3.1	8.4%	−18.3%
Total	**37.0**	100.0%	17.8%

Source: A1, A2, C1; shares derived [C18].

3. Management

Joe Burnett has been President and CEO since November 7, 2017. He joined when the company was still called MRI Interventions and became a director at the same time [D13]. He came from Royal Philips, where he was VP and GM of Neuro Diagnostics and Therapy from March 2016. Before that he was SVP of Image Guided Therapy Devices and GM of Volcano (a Philips company) in 2015–16 [D13]. He spent November 2004–February 2015 at Volcano, ending as EVP/GM of its Coronary & Systems unit. Earlier he was an R&D engineer and product manager at Guidant (1999–2004) [D13]. He holds a BSE in biomedical engineering and an MBA from Duke (Fuqua) [D13].

Track record. Revenue grew from $7.38M in 2017 to $37.0M in FY2025A, roughly a fivefold increase over eight years [A6][A1]. Burnett repositioned the company from a neurosurgery tool vendor into a drug-delivery partner for biopharma. That is the strategy this forecast depends on. His capital allocation has relied on equity and structured debt. He closed the IRRAS acquisition in November 2025 for $5.0M in cash plus 1,325,000 shares and an earnout [D1]. He also raised Oberland/TPC notes ($50.2M carried at June 30, 2026), which carry a revenue participation from 2027 [D2]. Execution is mixed. FY2025A revenue landed in the bottom fifth of its original range [C13]. The 2026 range was cut by $4M in August [G2]. On the Q2 call, Burnett said BDD fell about 15% against a one-time order in the prior year, and he announced a commercial reorganization [D3]. In his own framing, he sets out priorities in pillars and gives concrete per-procedure economics [G4][G5]. **Ownership:** we did not verify Burnett's shareholding (no proxy opened), so the record on alignment is thin.

4. Key Developments

Figure 3 lists the events that set our assumptions.

Figure 3 – Key developments

Date	Event	Source
Aug 2025	BlueRock exPDite-2 Phase 3 (about 102 patients) first patient; primary completion March 2027	[D10]
Sep 2025	Bayer expects Phase 3 results in 2028–29 (Reuters; snippet only)	[D19]
Sep 2025	uniQure sizes about 6,000 treatable US patients at launch	[G8]
Nov 20, 2025	IRRAS (IRRAflow) acquisition closed	[D1]
Mar 17, 2026	FY2025A revenue $37.0M; 2026 guidance $52.0–56.0M	[A1][D4]
Aug 3, 2026	Q2 2026A revenue $10.9M; guidance cut to $48.0–52.0M; Burnett gives procedure capacity	[A4][G2][G7]
Sep 2, 2026	uniQure submits AMT-130 BLA; with priority review, action date late Apr/early May 2027	[D9][S8]

The guidance cut shows slower core growth. The AMT-130 BLA gives a dated path to the procedure line, while bemdaneprocel's later readout pushes it to FY2029E.

5. Business Drivers by Line

- **Biologics and drug delivery, neurosurgery, capital equipment.** Trial enrolment (10–15 trials, about 10 readouts in 18 months) and IRRAflow drive FY2026E–FY2027E, with growth of 15% a year from FY2028E (low end of Burnett's 15–20% [G12]) [G2][S1]–[S7][S26].
- **CAL preclinical services (uplift inside biologics and drug delivery).** The CAL is ClearPoint's own preclinical contract-research lab, not a manufacturing plant [D1][D2]. The lease was signed in June 2025, and the last of three phases was handed over on July 13, 2026 [D2]. ClearPoint signed its first GLP (Good Laboratory Practice) statement of work there, worth "multi-million" dollars, and expects to complete it in H1 2027 [G10]. Burnett says the old subleased lab peaked at about $8M a year of preclinical work, and the CAL "could surpass $60 million or so" [G9]. He says one large GLP study could be worth $15–20M [G11]. The Q2 2026 miss was blamed on a slow CAL ramp, and 2026 guidance includes none of the large studies, which should start in H2 2027 [G9][G10]. Management also names CAL as one of the levers that could take 2027 above "high teens" growth [G9]. We model CAL as an uplift on top of the 15% core growth: nothing in FY2027E in the base (consensus already reflects the first contract), then $7M, $14M, $22M, $29M and $36M in FY2028E–32E, reaching 60% of capacity [S37] ★. 60% is a central reading of management's "could surpass $60 million" [G9]. The bull adds $3M in

FY2027E and reaches $45M (75% of capacity). The bear reaches only $10M [S37]. Revenue is booked as each study stage completes, so actual results will be lumpier than this smooth path [G10].

- **Commercial drug delivery (scenario).** This line is volume-driven, with price fixed at $20k [P2]. Volume is set by centre activation and cases per centre per week, using Burnett's capacity comments [G7]. AMT-130 first works through uniQure's 6,000 treatable pool [G8], then runs at a steady state of 3,000 a year, set as a share of HD incidence [S30]. The Parkinson's volume is the class of one-time Parkinson's cell/gene therapy procedures, not a single product: every such procedure is assumed to use ClearPoint's platform [S62][S63] ★. The first launch is bemdaneprocel (H2 2029E) [S20]. From launch, class volume ramps over three years to class demand: one third of demand in FY2029E, two thirds in FY2030E and all of it from FY2031E; class demand then governs [S31][S64][C40]. Figure 4a shows the ramp.

Steady-state rationale ★. *Huntington's:* US incidence is about 4,087 new cases a year (1.22/100k × 335M) [A7][C23]. Adding the UK (0.67/100k × 68M) brings it to about 4,543 [A8][C24]. 3,000 a year is 73% of the US figure, or 66% of US + UK [C25]. That is well above uniQure's 30% treatable share of *prevalent* patients [C21]. We accept it because newly diagnosed patients are, by definition, at the early-manifest stage AMT-130 was studied in, and there is no competing disease-modifying therapy. It is still aggressive, and it would be unreachable if access stays US-only and narrow. *Parkinson's:* see the eligible-pool summary below and Appendix C.

Parkinson's demand: eligible-pool model ★. We size demand for the class of one-time Parkinson's cell and gene therapies, not for bemdaneprocel alone, using label-style criteria modelled on DBS rather than trial entry criteria. The model starts from published US prevalence of about 1.10 million in FY2026E [A30][C41] and keeps patients who have had PD for at least four years and do not have dementia (45%) [A31][A32], have motor complications (50%, a judgment anchored to cohort data) [A33][A34][S46], have levodopa-responsive idiopathic PD (81%) [A35] and are fit for surgery (90%) [S47]. That leaves about 180,000 medically eligible patients (179,813), 16% of PD, close to the roughly 15% in two published references [C42][A36][A12]. Patients who have already had DBS stay in the pool. Every Parkinson's cell/gene trial we checked excludes them, but that is a trial-entry rule: no registry entry gives a biological or safety reason, DBS has been added after grafts in small case series, and grafts have been placed in three patients who already had DBS [A41]–[A43][S44] ★. Each year new patients enter, others leave through death or dementia, and from FY2029E 6% of the pool receives a one-time therapy [A31][A39][S45] ★. Nobody receives a one-time therapy before launch, and entrants of (m + g) × the starting pool keep the pool growing at the 2.9% prevalence rate to FY2028E [C45]. The 6% is anchored on today's DBS rate: 3,990 DBS procedures a year are 2.2% of the 180,000 pool [A10][C49]. We assume about 2.7 times as many patients a year would choose a one-time, potentially disease-modifying therapy as choose DBS, which is symptomatic hardware surgery. That multiple is our judgment; no published willingness study supports it [S45]. DBS continues alongside: it is not deducted from one-time demand, and patients who have had DBS remain eligible [S44]. Appendix C gives the full method, sources and weak points.

Figure 4b shows class demand and the three-year ramp to it in the base.

Figure 4b – One-time Parkinson's therapy class demand and ClearPoint class volume (base case)

Patients a year	**FY2029E**	**FY2030E**	**FY2031E**	**FY2032E**	**FY2033E**	**FY2035E**	**FY2040E**
Eligible pool, start of year (DBS patients included)	196,000	190,000	185,000	182,000	181,000	180,000	191,000
One-time therapy class demand (6% of pool)	**11,756**	**11,391**	**11,126**	**10,945**	**10,837**	**10,801**	**11,454**
Memo: DBS (context; not deducted)	3,990	3,990	3,990	3,990	3,990	3,990	3,990
Ramp share of class demand	1/3	2/3	100%	100%	100%	100%	100%
Class volume on ClearPoint	3,919	7,594	11,126	10,945	10,837	10,801	11,454
Implied centres at 3 cases/wk × 50 wks	26	51	74	73	72	72	76

Source: A10, A30–A35, A39, S31, S43–S45, S62–S64, C40, C42, C45; Appendix C. FY2035E and FY2040E show the class path for context; Finagle extends revenue after FY2032E by its growth fade, not by this path. Pool rounded to the nearest thousand. Centres derived.

What this implies. In the base, demand, not capacity, sets volume from FY2031E. Class demand peaks in the launch year, because 6% is applied to a pool that has grown untouched to FY2028E. It then eases by about 8% to FY2034E as that backlog is drawn down, and grows again with prevalence, about 0.8% a year over FY2033E–40E (derived) [C40]. Under the three-year ramp the launch-year peak is never reached in volume: FY2029E–30E volume is below demand, and from FY2031E volume equals demand. David's reason for three years is that this also gives time for other Parkinson's therapies to be approved [S64] ★. That rationale is a judgment, not a timeline match: bemdaneprocel launches in FY2029E, but the

other quantified Parkinson's therapies are not expected before FY2033E–34E (Appendix D), after the ramp has ended. For FY2031E–32E the model therefore has the first product, or the platform generally, meeting all class demand. Because the line is the whole class, no product split is needed (Appendix D shows an illustrative split). *Bull and bear.* The bull keeps the base pool and the three-year ramp and raises the share treated to 8% a year [S59] ★ (Figure 4c). The bear applies the trial-style exclusion: patients who have had DBS (about 42,000, i.e. 3,990 a year ÷ the 0.095 leave rate) are removed, leaving about 138,000, and 6% of that gives class demand of about 8,400 in FY2032E [C55]. Approval slips to 2030, and a capacity ramp runs to the bear capacity of 4,500 in FY2032E, which stays the limit in every year, including after FY2032E [S22][S43].

Figure 4c – Bull case: class demand with a higher treated share (8%)

Patients a year	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2035E	FY2040E
Eligible pool, start of year (DBS patients included)	196,000	186,000	178,000	173,000	169,000	166,000	172,000
One-time therapy class demand (bull, 8% of pool)	**15,674**	**14,875**	**14,272**	**13,833**	**13,530**	**13,250**	**13,792**
Class volume on ClearPoint (three-year ramp)	5,225	9,917	14,272	13,833	13,530	13,250	13,792
Implied centres at 3 cases/wk × 50 wks	35	66	95	92	90	88	92
Memo: base class volume	3,919	7,594	11,126	10,945	10,837	10,801	11,454

Source: A30–A35, A39, S21, S59, S62–S64, C45, C46; Appendix C. Same pool inputs as the base; only the treated share changes. Pool rounded to the nearest thousand. Centres derived.

With 8% treated, the pool settles at a steady 39% of each year's entrants treated, against 33% at 6% (derived) [C46]. 8% is a third above the base rate and about 3.6 times the 2.2% DBS rate [C49]. We chose the smallest change that keeps the bull clearly above the base in every year: a bull defined by a capacity build to 10,000 a year would now sit below the base, which exceeds 10,000 from FY2031E. With the base already on a three-year ramp, the bull differs from it only in the treated share [S59] ★. Bull volume is 14,300 in FY2031E and 13,800 in FY2032E, against 11,100 and 10,900 in the base. David earlier asked for a bull growing to 10,000 a year; this bull exceeds that, which needs his confirmation [S59].

Capacity is not the limit in the base or bull. S20 assumes 4,500 a year per therapy (30 centres × 3 a week × 50 weeks). Burnett's "20–30 centres … 50 to 100 cases a week" [G7] reaches 5,000 a year at the top end. Base Parkinson's volume of about 11,000 a year from FY2031E needs about 74 centres at 3 cases a week, and the bull's 14,300 about 95, on top of AMT-130's 3,000 (about 20 more centres if the networks are separate) (derived) [S43] ★. That is two and a half to three times the top of management's comments. We accept it as an aggressive but bounded judgment: volume is driven by the eligible pool (6% of about 180,000), not by a capacity plan, and the three-year ramp implies adding about 25 centres a year in the base (about 30 in the bull). Limiting the ramp by an explicit build of 25 centres a year would give almost the same path, so we keep the simpler rule [S64]. Burnett's quote may cover all therapies. Launch analogs such as Hemgenix and Roctavian were slow [D22][D23], but those are small rare-disease markets without a surgical delivery network. AMT-130 still follows the S20 capacity ramp: 330 → 2,500 → 3,170 → 3,000 [S32]. The bear remains limited by its 4,500 capacity [S43].

Figure 4a – Procedures by therapy (base case)

Procedures	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E
AMT-130	330	2,500	3,170	3,000	3,000	3,000
One-time PD therapies (class)	–	–	3,919	7,594	11,126	10,945
Total	**330**	**2,500**	**7,089**	**10,594**	**14,126**	**13,945**
Procedure revenue at $20k (US$M)	6.6	50.0	141.8	211.9	282.5	278.9
CAL uplift (US$M)	–	7.0	14.0	22.0	29.0	36.0
Binding constraint	AMT capacity	AMT capacity	AMT pool / PD ramp	PD ramp	PD class demand	PD class demand

Source: S20, S30–S32, S37, S62–S64, G7, G8, G9, P2. The PD row is the class of one-time Parkinson's cell/gene therapy procedures on ClearPoint's platform; bemdaneprocel is the first launch.

6. Risks

Company-specific.

- **Capacity vs. demand.** Our volumes assume ClearPoint's stated capacity is fully used. Gene-therapy launches have often been slow: Hemgenix treated 12 patients in its first year [D22], and Roctavian 3 in 2023 [D23]. Our bear case models this slower path: 270 AMT-130 procedures in FY2028E [S22]. FY2028E revenue is then $54.2M below the base (derived). The bear, which also delays bemdaneprocel and slows core growth, is valued at $16.72 a share [C16].
- **Shared centres.** AMT-130 and the Parkinson's class would compete for the same neurosurgeons and MRI suites; base FY2032E volume of about 13,900 procedures needs about 93 centres at 3 cases a week (derived) [S43].
- **Funding and dilution.** Cash was $29.4M at June 30, 2026 vs. $50.2M of notes [D2]. Finagle's lowest modelled cash is −$41M (base) to −$157M (bear) [C14][C16], so an equity raise is likely and is not modelled.
- **Steady-state share of incidence.** The 3,000-a-year AMT-130 figure needs about two-thirds of US + UK new HD diagnoses treated every year [C25] ★. Each 500 fewer procedures takes $10M off revenue (derived).
- **Parkinson's pool and uptake.** Class demand rests on two judgments, the 50% motor-complication share and 90% fitness; on a 6% annual treatment rate, about 2.7 times today's 2.2% DBS rate, which no published willingness study supports; and on keeping patients who have had DBS in the pool [S44]–[S47][C49] ★. Every trial we checked excludes prior DBS; if labels did the same, the pool would be about 138,000 and base class demand about 8,400 a year, as in the bear [A41][C55]. Volume equals class demand from FY2031E, so any shortfall in the pool or in uptake cuts volume directly. Each 1,000 fewer procedures a year takes $20M off revenue and about $2.69 off the share value if it starts in FY2030E (derived) [C43].
- **Centre build.** The base needs about 74 centres for Parkinson's alone from FY2031E, against management's 20–30 [G7][S43] ★. If ClearPoint and its partners cannot staff that many, capacity rather than demand becomes the limit, as in the bear.
- **Platform share (100%).** We assume every one-time Parkinson's cell/gene procedure uses ClearPoint's platform. This is David's judgment [S63] ★. His reasoning: a sponsor with a multi-billion-dollar therapy will not risk its label or approval to save a few thousand dollars on a cannula that was not used in the pivotal trial. The delivery device used in the trial becomes part of the approved procedure, so once a therapy is approved with ClearPoint's system, ClearPoint's share of that therapy stays near 100%.
    - *Trial evidence consistent with this.* Bemdaneprocel's Phase 1 used "a modified cannula (Smart Flow, Clearpoint Neuro)" [D12]. Aspen says ASPIRO (ANPD001) uses the ClearPoint system "for all patients" [D68]. We could not confirm the delivery device for exPDite-2, REGENERATE-PD (AB-1005), RNDP-001 or the NIH AAV2-GDNF Phase 1 from public registry entries [D10]. This evidence fits David's reasoning but does not prove that later approvals will specify ClearPoint.
    - *Brainlab.* Brainlab sells planning and navigation software for catheter placement: iPlan Flow ("a dedicated tool for planning trajectories of intra-cranial catheters", 510(k) K053164) and cranial navigation (K223288, K223552) [D63]. Its "Flexible Catheter" is listed with the FDA under ClearPoint's own 510(k) K123605 (filed as MRI Interventions), which clears the SmartFlow Flex Ventricular Catheter "for injection of Cytarabine or removal of CSF from the ventricles" (product code HCA) [D64]. A 2021 study in diffuse intrinsic pontine glioma (DIPG) used that catheter to infuse brain-stem tissue, outside that ventricular indication, and placed it with the ClearPoint system [D65].
    - *Renishaw / Neurochase.* Neuroinfuse is a bone-anchored transcutaneous port for repeated, intermittent infusions over months or years through several cannulas, with "an MRI compatible application set" [D66]. In the Bristol GDNF trial it gave monthly infusions [D67]. It is investigational and "can only be used in the setting of an approved clinical trial" [D66]. Renishaw agreed to sell neuroinfuse and all of its IP to Neurochase, with transfers due to complete by January 2026 [D69]. Neurochase says the system has been used under humanitarian approval for chemotherapy in children with brain-stem tumours, and it planned a DIPG trial for early 2026 [D69]. We found no record that this trial has started and no regulatory clearance for neuroinfuse. Neurochase's own nFuse system is "for investigational use only. Not approved for clinical use in any geography" [D70].
    - *Conclusion and residual risk.* We found no commercially cleared, MRI-guided intraparenchymal CED cannula designed by either company. ClearPoint's 10-K still says Brainlab and Renishaw "offer systems such as navigational platforms and cannulas useful for drug delivery under MRI. These offerings are competitive with our products." [D1] Two risks remain. First, other navigation platforms: one bemdaneprocel Phase 1 site placed the cannula with "a frame-based stereotactic approach" [D12]. Second, future devices, such as Neurochase's, which is designed to work with existing robots and frames [D70].
    - *Sizing.* Each 1,000 procedures a year lost from FY2030E (about 9% of base FY2032E Parkinson's volume) is about $20M of revenue and $2.69 a share (derived, base perpetuity) [C43].

- **No supply agreement disclosed** with uniQure, BlueRock or the other Parkinson's partners. One bemdaneprocel Phase 1 site used a different, frame-based approach [D12].

Industry and regulatory.

- **Approval timing.** Without priority review, or with a request for more data, AMT-130 slips by a year or more. Bayer's 2028–29 readout makes a 2029 bemdaneprocel approval the earliest plausible date [D19].
- **Pricing and reimbursement.** Our $20k per procedure is set by the prompt [P2]. It sits inside management's $15–25k range [G4] but above the $12–15k partner average [G5].

7. Forecast

How the evidence becomes the numbers

Totals for FY2026E and the FY2027E core come from consensus, because guidance was revised [E1][G2]. The core lines were scaled up by about 7% (H2 2026E) and 4% (FY2027E) to match those totals [C4][C6]. Core growth after FY2027E is our judgment [S7][S26]. The procedure line is a volume build at $20k, added on top: AMT-130 follows a capacity ramp and the Parkinson's class a three-year ramp to class demand [P2][S19][S64]. The CAL uplift is also added on top of core growth [S37]. EBITDA is a cost build, because there is no EBITDA guidance or consensus [C9]. Figure 4 maps each piece of evidence to the assumption it sets.

Figure 4 – Evidence → assumption → forecast impact

Evidence	Assumption	Forecast impact
Guidance $48.0–52.0M [G2]; consensus $49.0M / $56.6M [E1]	Totals FY2026E and FY2027E core = consensus	Revenue $49.0M FY2026E
Guidance excludes commercial drug delivery [G3]	Procedure line added on top [S19]	Double-count risk
15 centres at 1–2/wk; 20–30 centres at 50–100/wk [G7]	AMT-130 ramp 330 → 2,500 → 4,500 cap [S20] ★; PD class needs about 74 centres (base) [S43] ★	Volumes
David: all procedures $20k [P2]; range $15–25k [G4]	$20k	Price
About 6,000 treatable at launch [G8]; HD incidence 1.22/100k US, 0.67/100k UK [A7][A8]	Pool used up in FY2029E; then 3,000 a year = 73% of US / 66% of US + UK incidence [S30][C25] ★	AMT-130 3,170 → 3,000
PD prevalence ≥930,000 (2020) to 1,238,000 (2030) [A30]; mortality, dementia, motor complications, diagnostic accuracy [A31]–[A35]; DBS 3,990 a year [A10]; trials exclude prior DBS, no biological reason found [A41]–[A43]	Medically eligible pool about 180,000, DBS patients included [C42][S44] ★; DBS 2.2% of it a year [C49]; 6% a year one-time from FY2029E (about 2.7× DBS) ★, DBS not deducted → class demand 10,800–11,800 [C40][S45]; all on ClearPoint [S63] ★; three-year ramp to demand [S64] ★	3,919 / 7,594 / 11,126 / 10,945 FY2029E–32E, 10,837 in FY2033E (bull 8%: 5,225 → 13,833, then 13,530 [C46]; bear 4,500 capacity, class demand about 8,400 [C55])
CAL handed over Jul 2026; first GLP contract; capacity >$60M vs. ~$8M [D2] [G9][G10]	CAL uplift $7M → $36M FY2028E–32E [S37]; 20% incremental opex [S38] ★	+$36.0M revenue, +$15.4M EBITDA in FY2032E (derived) [C35]
Core 15%; CAL increment; PD volume already at class demand [S26][S37] [S64]	FY2033E growth (gsnext) 5.1%, fading linearly to Finagle's default by FY2045E [C29][S29]	Small FY2033E change in PD volume (−108) [C29]
AMT-130 action date late Apr/early May 2027 [D9]	Launch Q3 2027 [S8]	330 in FY2027E
Bayer readout 2028–29 [D19]; gene-therapy launches slow [D22][D23]	First class launch (bemdaneprocel) H2 2029 [S20]; three-year ramp to class demand [S64] ★	3,919 FY2029E, 7,594 FY2030E, 11,126 FY2031E, 10,945 FY2032E
Disposables "high margin" [D1]	Procedure gross margin 70% [S23] ★	Margin rises to about 68%
Specialists and field support needed	10% of revenue + $2M per therapy [S24][S25] ★	Procedure opex $31.9M in FY2032E (derived)
"cash burn to decrease in the second half" [G6]	Core opex −2% in H2, +6% in FY2027E, +5% after [S14]–[S16]	EBITDA path

Source: tags as shown. ★ = judgment call.

Base case

Figure 5 shows the procedure line reaching 61% of revenue in FY2029E and 65% in FY2032E, with CAL at 8% (derived). Growth peaks in FY2028E–29E as AMT-130 and then the Parkinson's class launch, then slows to 4% in FY2032E as Parkinson's demand eases after the launch backlog.

Figure 5 – Base case by line

US$M	FY2024A	FY2025A	H1 2026A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E
Biologics and drug delivery	17.3	19.0	8.8	19.7	22.8	26.3	30.2	34.7	40.0	45.9
Neurosurgery navigation and therapy	10.3	14.8	11.4	23.7	27.5	31.7	36.4	41.9	48.2	55.4
Capital equipment and software	3.8	3.1	2.7	5.7	6.2	7.1	8.2	9.4	10.8	12.4
Commercial drug delivery (AMT-130, Parkinson's class) [scenario]	–	–	–	–	6.6	50.0	141.8	211.9	282.5	278.9
Preclinical services uplift, CAL GLP facility [scenario]	–	–	–	–	–	7.0	14.0	22.0	29.0	36.0
Total revenue	**31.4**	**37.0**	**23.0**	**49.0**	**63.1**	**122.0**	**230.6**	**319.9**	**410.4**	**428.6**
Growth	–	17.8%	30.0%	32.5%	28.9%	93.2%	88.9%	38.7%	28.3%	4.4%
EBITDA (before SBC)	–	−15.2	−13.0	−23.6	−19.5	12.0	71.2	120.6	170.8	177.5
EBITDA margin	–	−41.0%	−56.5%	−48.1%	−30.9%	9.8%	30.9%	37.7%	41.6%	41.4%

Source: A1–A4, C2, C5, C7, C9, C10, S1–S7, S15, S20–S26, S30–S32, S37, S38, P2. CAL revenue is reported inside biologics and drug delivery; it is shown separately here. May not sum due to rounding.

Valuation (Finagle)

Method. Finagle values equity as free cash flow to equity (FCFE) discounted at the cost of equity [C14][S29]. FCFE is EBITDA less tax, capex and debt flows. It takes our FY2026E–FY2032E revenue and EBITDA as given (seven explicit years). We use a 20-year horizon (FY2045E), so Finagle extends the forecast by 13 years [S29]. Finagle sets FY2033E revenue growth to gsnext and fades it in a straight line to its default terminal growth rate by year 20. Each extra dollar of revenue earns EBITDA at the incremental margin mc. A Gordon-growth terminal value is added after FY2045E. We set gsnext to the growth the business would show in FY2033E: core at 15%, one more year of the CAL increment, and the change in Parkinson's volume from FY2032E to FY2033E class demand (base −108, bull −303 procedures; the bear stays at its 4,500 capacity), plus the bear's AMT-130 step from 2,700 to 3,000 [C29]. Base growth glides from 5.1% to 5.0% over 13 years (Figure 8a). Finagle accepts any whole-number horizon of one year or more [S29]. Negative FCFE is treated as a negative dividend, i.e. funded by shareholders with no new shares [C14]. We never set terminal growth; Finagle's default (4.95%) applies [S29].

Figure 7 – Valuation inputs

Input	Bear	Base	Bull	Source
FY2032E EBITDA margin (me)	22.7%	41.4%	44.5%	C12
Incremental margin after FY2032E (mc) ★	45%	50%	55%	S17
FY2033E revenue growth (gsnext)	6.2%	5.1%	5.1%	C29
Horizon (years) / extended years	20 / 13	20 / 13	20 / 13	S29
Terminal growth (gt, Finagle default)	4.95%	4.95%	4.95%	C14
Cost of equity (re, Finagle default)	10.45%	10.45%	10.45%	C14
Cost of debt / tax rate	8.25% / 21%	8.25% / 21%	8.25% / 21%	S29
Terminal WACC (Finagle output)	10.4%	10.4%	10.4%	C14–C16
Debt / cash, FY2025A (US$M)	49.1 / 45.9	49.1 / 45.9	49.1 / 45.9	D1
Federal NOL (US$M)	258.2	258.2	258.2	D1
Shares (M) / price (US$)	30.50 / 9.25	30.50 / 9.25	30.50 / 9.25	D2, C17

Source: tags as shown. ★ = judgment call. mc reflects procedure gross margin less specialist cost, blended with core [S17]. gsnext is derived from the FY2032E line mix [C29]. In the base and bull, Parkinson's volume dips slightly in FY2033E as class demand eases, so gsnext is close to Finagle's 4.95% default; the bear's is higher because AMT-130 adds 300 procedures on the smallest revenue base [C29].

Figure 8a – Revenue growth path, FY2027E–FY2045E (explicit to FY2032E, Finagle fade after)

Revenue growth	2027	2028	2029	2030	2031	2032	2033	2034	2035	2036	2037	2038	2039	2040	2041	2042	2043	2044	2045
Bear	16.7%	21.2%	31.6%	33.4%	51.2%	31.1%	6.2%	6.1%	6.0%	5.9%	5.8%	5.7%	5.6%	5.5%	5.4%	5.3%	5.2%	5.1%	5.0%
Base	28.9%	93.2%	88.9%	38.7%	28.3%	4.4%	5.1%	5.1%	5.1%	5.1%	5.1%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%
Bull	55.4%	115.4%	69.9%	36.8%	29.0%	4.0%	5.1%	5.1%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%

Source: Finagle revenue path, derived [C34]. Growth in each later year approaches the 4.95% default; Finagle applies that rate after FY2045E.

Figure 8a shows base growth of 88.9% in FY2029E as the Parkinson's class launches, then 38.7%, 28.3% and 4.4% in FY2032E, 5.1% in FY2033E and about 5.0% to FY2045E.

Figure 8 – Value bridge (US$)

	Bear	Base	Bull
PV of FCFE, FY2026E–FY2045E, per share	4.35	27.90	38.19
PV of terminal value, per share	12.37	26.39	34.50
Value per share	**16.72**	**54.29**	**72.68**
Terminal value share of total	74%	49%	47%
Firm value (US$M)	513.2	1,659.1	2,220.2
Less debt (US$M)	49.1	49.1	49.1
Equity value (US$M)	464.1	1,610.0	2,171.1
Year-20 (FY2045E) revenue / EBITDA (US$M)	480.3 / 163.5	812.0 / 369.2	986.5 / 487.9
Terminal-year FCFE (US$M)	133.1	304.9	404.2
Lowest modelled cash (US$M)	−157.4	−41.2	−27.9
First year of positive EBITDA	FY2031E	FY2028E	FY2028E

Source: C14–C16, C27 (Finagle valuation output). The FCFE PV discounts Finagle's per-share dividend stream at re. The terminal value is the residual to Finagle's value per share (derived). Firm value less debt = equity (derived). The bear's PV of FCFE is small because of its losses before FY2031E.

Figure 9 – Finagle valuation table, base case (layout of Finagle's Excel template)

Header (report sheet)	Value
Ticker	CLPT
Price (US$)	9.25
Cost of Debt	8.2%
Cost of Equity	10.45%
Terminal growth (default)	4.95%
Marginal Tax	21.0%
Cash Generated (US$M)	3,128.8
Terminal FCFE (US$M)	304.9
Terminal Value (US$M)	5,543.1
Shares (M)	30.50
Intrinsic Value per Share (US$)	54.29
IV per share DDM model (US$)	54.29

FY2025A–FY2035E

US$M	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	37.0	49.0	63.2	122.0	230.6	319.9	410.4	428.6	450.5	473.5	497.6
EBITDA	−15.2	−23.6	−19.5	12.0	71.2	120.6	170.8	177.5	188.4	199.9	212.0
Stock-based compensation	8.2	12.7	10.5	−6.5	−38.4	−65.0	−92.1	−95.7	−101.6	−107.8	−114.3
Interest	2.4	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0
Pre-tax Income	−26.6	−41.4	−35.5	11.7	100.5	174.5	249.8	259.7	276.1	293.2	311.2
Net Operating Loss (NOL)	258.2	299.6	335.1	323.4	222.9	48.3	0.0	0.0	0.0	0.0	0.0
Cash Taxes	0.1	0.0	0.0	0.0	0.0	0.0	42.3	54.5	58.0	61.6	65.4
Capex	0.5	3.0	6.0	18.2	40.7	59.3	78.1	81.8	86.4	91.2	96.2
MnA	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
dWC	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
dDebt	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
FCF, 'Owners Earnings'	−26.3	−43.3	−40.1	−3.8	64.8	122.3	138.5	132.8	141.6	150.9	160.7
FCFE	−26.3	−43.3	−40.1	−3.8	64.8	122.3	138.5	132.8	141.6	150.9	160.7
FCFF	−24.4	−40.1	−36.9	−0.6	68.0	125.5	141.7	136.0	144.8	154.1	163.9
Firm, Intrinsic Value	1,659.1	1,870.7	2,101.2	2,319.4	2,491.9	2,625.0	2,755.7	2,905.9	3,062.9	3,227.0	3,398.5
Debt, Market Value	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1
Enterprise Value	1,659.1	1,870.7	2,101.2	2,319.4	2,491.9	2,625.0	2,755.7	2,905.9	3,062.9	3,227.0	3,398.5
Equity, Intrinsic Value	1,610.0	1,821.6	2,052.1	2,270.4	2,442.8	2,575.9	2,706.7	2,856.8	3,013.8	3,177.9	3,349.4
Cash	45.9	2.6	−37.5	−41.2	23.6	145.9	284.4	417.1	558.8	709.7	870.4
Cash (on balance-sheet)	45.9	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Revenue growth (derived)	–	32.5%	28.9%	93.2%	88.9%	38.7%	28.3%	4.4%	5.1%	5.1%	5.1%
FCF per share (US$)	−0.86	−1.42	−1.31	−0.12	2.13	4.01	4.54	4.35	4.64	4.95	5.27
Dividend in DDM model (US$/sh)	−0.86	−1.42	−1.31	−0.12	2.13	4.01	4.54	4.35	4.64	4.95	5.27
Cost of Capital, WACC	10.3%	10.3%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%
Discount factor at re (derived)	–	0.905	0.820	0.742	0.672	0.608	0.551	0.499	0.451	0.409	0.370
PV of dividend (US$/sh, derived)	–	−1.29	−1.08	−0.09	1.43	2.44	2.50	2.17	2.10	2.02	1.95
Intrinsic Value per Share (US$)	54.29	59.72	67.27	74.43	80.09	84.45	88.73	93.66	98.80	104.18	109.81

FY2036E–FY2045E

US$M	FY2036E	FY2037E	FY2038E	FY2039E	FY2040E	FY2041E	FY2042E	FY2043E	FY2044E	FY2045E
Revenue	522.8	549.3	577.0	606.1	636.6	668.5	701.9	736.9	773.6	812.0
EBITDA	224.6	237.8	251.7	266.2	281.4	297.4	314.1	331.6	350.0	369.2
Stock-based compensation	−121.1	−128.2	−135.7	−143.5	−151.8	−160.4	−169.4	−178.8	−188.7	−199.1
Interest	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0	4.0
Pre-tax Income	330.1	349.9	370.6	392.4	415.1	439.0	464.0	490.2	517.6	546.3
Net Operating Loss (NOL)	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Cash Taxes	69.3	73.5	77.8	82.4	87.2	92.2	97.4	102.9	108.7	114.7
Capex	101.4	106.9	112.7	118.7	125.0	131.6	138.6	145.9	153.5	161.4
MnA	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
dWC	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
dDebt	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
FCF, 'Owners Earnings'	170.9	181.6	192.9	204.6	217.0	229.9	243.4	257.6	272.5	288.0
FCFE	170.9	181.6	192.9	204.6	217.0	229.9	243.4	257.6	272.5	288.0
FCFF	174.1	184.8	196.1	207.8	220.2	233.1	246.6	260.8	275.7	291.2
Firm, Intrinsic Value	3,577.7	3,765.0	3,960.5	4,164.8	4,378.0	4,600.6	4,833.0	5,075.4	5,328.4	5,592.2
Debt, Market Value	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1	49.1
Enterprise Value	3,577.7	3,765.0	3,960.5	4,164.8	4,378.0	4,600.6	4,833.0	5,075.4	5,328.4	5,592.2
Equity, Intrinsic Value	3,528.6	3,715.9	3,911.4	4,115.7	4,328.9	4,551.5	4,783.9	5,026.3	5,279.3	5,543.1
Cash	1,041.3	1,222.9	1,415.8	1,620.4	1,837.4	2,067.2	2,310.7	2,568.3	2,840.7	3,128.8
Cash (on balance-sheet)	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Revenue growth (derived)	5.1%	5.1%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%
FCF per share (US$)	5.60	5.95	6.32	6.71	7.11	7.54	7.98	8.45	8.93	9.44
Dividend in DDM model (US$/sh)	5.60	5.95	6.32	6.71	7.11	7.54	7.98	8.45	8.93	9.44
Cost of Capital, WACC	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%	10.4%
Discount factor at re (derived)	0.335	0.303	0.275	0.249	0.225	0.204	0.184	0.167	0.151	0.137
PV of dividend (US$/sh, derived)	1.88	1.81	1.74	1.67	1.60	1.54	1.47	1.41	1.35	1.29
Intrinsic Value per Share (US$)	115.68	121.82	128.23	134.93	141.92	149.22	156.83	164.78	173.07	181.72

Source: Finagle valuation of the base case, laid out in Finagle's standard company template [C30]. Rows and labels follow the template; the growth, discount factor and PV rows are added and derived. Each year's "Equity, Intrinsic Value" is Finagle's equity value at that date; the FY2045E value is the template's "Terminal Value". The SBC row is printed as Finagle computes it, not adjusted by us. Cash taxes stay near zero until the NOL is used up in FY2031E.

Figure 10 – Finagle valuation table, key rows, bull and bear

Bull

FY2025A–FY2035E

US$M	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	37.0	50.0	77.7	167.4	284.4	389.2	502.1	522.3	548.7	576.5	605.6
EBITDA	−15.2	−22.9	−11.3	38.8	103.0	161.5	225.0	232.6	247.2	262.4	278.4
Cash Taxes	0.1	0.0	0.0	0.0	0.0	24.0	69.5	71.9	76.5	81.3	86.3
Capex	0.5	3.4	9.6	29.6	55.6	78.9	104.1	108.6	114.5	120.7	127.1
FCFE	−26.3	−42.8	−31.0	26.0	98.8	141.6	168.6	173.5	185.4	197.9	211.0
Equity, Intrinsic Value	2,171.1	2,440.8	2,727.0	2,986.0	3,199.3	3,392.1	3,578.1	3,778.6	3,988.2	4,207.2	4,435.9
Cash	45.9	3.2	−27.9	−1.8	97.0	238.6	407.2	580.7	766.1	964.0	1,175.0
Revenue growth (derived)	–	35.2%	55.4%	115.4%	69.9%	36.8%	29.0%	4.0%	5.1%	5.1%	5.0%
Intrinsic Value per Share (US$)	72.68	80.02	89.40	97.89	104.88	111.20	117.30	123.88	130.75	137.93	145.43

FY2036E–FY2045E

US$M	FY2036E	FY2037E	FY2038E	FY2039E	FY2040E	FY2041E	FY2042E	FY2043E	FY2044E	FY2045E
Revenue	636.1	668.1	701.7	736.8	773.7	812.4	852.9	895.3	939.8	986.5
EBITDA	295.2	312.8	331.3	350.6	370.9	392.2	414.4	437.8	462.3	487.9
Cash Taxes	91.6	97.2	103.0	109.1	115.5	122.2	129.2	136.5	144.2	152.3
Capex	133.9	141.1	148.5	156.4	164.6	173.2	182.2	191.7	201.6	212.0
FCFE	224.8	239.2	254.3	270.2	286.8	304.2	322.5	341.6	361.7	382.7
Equity, Intrinsic Value	4,674.8	4,924.3	5,184.7	5,456.5	5,740.0	6,035.8	6,344.2	6,665.8	7,000.9	7,350.0
Cash	1,399.8	1,639.0	1,893.3	2,163.5	2,450.3	2,754.5	3,077.0	3,418.7	3,780.3	4,163.0
Revenue growth (derived)	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%	5.0%
Intrinsic Value per Share (US$)	153.26	161.44	169.97	178.88	188.18	197.88	207.99	218.53	229.51	240.96

Bear

FY2025A–FY2035E

US$M	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	37.0	48.0	56.0	67.9	89.3	119.1	180.2	236.3	250.9	266.1	282.0
EBITDA	−15.2	−24.7	−25.2	−24.5	−16.4	−5.5	25.8	53.7	60.3	67.1	74.3
Cash Taxes	0.1	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Capex	0.5	2.3	3.7	5.6	9.1	14.0	24.1	33.3	35.7	38.2	40.8
FCFE	−26.3	−44.5	−46.5	−47.4	−38.4	−26.6	11.5	45.3	53.0	61.0	69.5
Equity, Intrinsic Value	464.1	557.1	661.8	778.4	898.1	1,018.6	1,113.5	1,184.7	1,255.5	1,325.7	1,394.8
Cash	45.9	1.5	−45.0	−92.4	−130.8	−157.4	−145.8	−100.6	−47.6	13.5	82.9
Revenue growth (derived)	–	29.8%	16.7%	21.2%	31.6%	33.4%	51.2%	31.1%	6.2%	6.1%	6.0%
Intrinsic Value per Share (US$)	16.72	18.26	21.70	25.52	29.44	33.39	36.51	38.84	41.16	43.46	45.73

FY2036E–FY2045E

US$M	FY2036E	FY2037E	FY2038E	FY2039E	FY2040E	FY2041E	FY2042E	FY2043E	FY2044E	FY2045E
Revenue	298.7	316.0	334.0	352.7	372.2	392.3	413.3	434.9	457.2	480.3
EBITDA	81.8	89.6	97.7	106.1	114.9	123.9	133.3	143.1	153.1	163.5
Cash Taxes	13.8	26.6	29.2	31.8	34.6	37.4	40.3	43.4	46.5	49.8
Capex	43.6	46.4	49.4	52.5	55.7	59.0	62.4	66.0	69.7	73.5
FCFE	64.4	60.7	67.7	75.0	82.5	90.3	98.4	106.8	115.4	124.4
Equity, Intrinsic Value	1,476.2	1,569.8	1,666.1	1,765.3	1,867.4	1,972.2	2,080.0	2,190.6	2,304.2	2,420.6
Cash	147.3	208.1	275.8	350.8	433.3	523.6	622.0	728.8	844.2	968.6
Revenue growth (derived)	5.9%	5.8%	5.7%	5.6%	5.5%	5.4%	5.3%	5.2%	5.1%	5.0%
Intrinsic Value per Share (US$)	48.40	51.46	54.62	57.87	61.22	64.66	68.19	71.82	75.54	79.36

Source: Finagle valuations of the bull and bear cases, standard company template [C30].

Figure 11 – Sensitivity of value per share (US$), one input at a time

Change	Bear	Base	Bull
As modelled	16.72	54.29	72.68
Cost of equity −1 pt	22.84	69.15	92.38
Cost of equity +1 pt	12.62	44.12	59.20
mc −10 pts	14.66	51.32	69.12
mc +10 pts	18.79	57.26	76.24
gsnext −5 pts	10.26	41.61	55.79
gsnext +5 pts	25.34	70.99	95.00
10-year horizon	18.15	52.33	70.04

Source: Finagle reruns of copies of each case, Oct 25, 2026 [C28]. Terminal growth is never set; it stays at the default.

gsnext is the most powerful single input (base $41.61–$70.99), because its fade runs for 13 years. A 10-year horizon cuts the base to $52.33, so the 20-year horizon adds about $1.96 a share in the base (derived); the gap is small because growth after FY2032E is already close to the terminal rate. In the bear the 10-year horizon gives slightly more ($18.15 vs. $16.72) [C28].

How the volume path interacts with gsnext. Finagle extends revenue after FY2032E only through gsnext, which it applies to all revenue and fades over 13 years. A one-year step in volume between FY2032E and FY2033E would therefore be compounded as if the whole business grew faster for a decade. For that reason volume reaches its long-run level within the explicit years in every case: class demand from FY2031E in the base and bull, and the 4,500 capacity in FY2032E in the bear. The FY2033E change in Parkinson's volume is small and slightly negative (base −108, bull −303), so gsnext carries no artificial step [C29].

Where the value comes from. Figure 8c separates the effect of the larger pool from that of the ramp to class demand [C48].

Figure 8c – Base value per share by step (US$)

	Bear	Base	Bull
Step	Value per share	gsnext	
Pool of 138,000 (prior-DBS patients excluded), PD volume held at 5,000 capacity	39.25	7.8%	
Pool of 180,000 (prior-DBS patients included), PD volume held at 5,000 capacity	39.25	7.8%	
Pool of 180,000, three-year ramp to class demand (as modelled)	54.29	5.1%	

Source: Finagle valuations, Oct 25, 2026 [C14][C48]. With volume held at capacity, the larger pool changes no value, because capacity binds either way; the uplift comes from letting volume follow class demand of about 11,000 a year from FY2031E.

The uplift of about $15 is roughly what the Figure 13 yardstick implies: about 6,000 extra procedures a year from FY2031E at about $2.5 a share per 1,000 (derived) [C43]. It is therefore not an artefact of Finagle's growth fade.

Figure 12 – Scenarios vs. price

	Bear	Base	Bull
Value per share (US$)	16.72	54.29	72.68
Price, Oct 23, 2026 (US$)	9.25	9.25	9.25
Difference vs. price	+80.8%	+486.9%	+685.7%

Source: C14–C17. Analysts' average target is $23.50 (2 analysts) [E2], shown for context only.

The spread mainly reflects core growth, CAL utilisation, approval timing and Parkinson's volume (4,500 vs. 10,900 vs. 13,800 a year in FY2032E). On these inputs the $9.25 price is below the bear value, but every case assumes both approvals [P1].

Value of additional procedure revenue

Each additional $20M of annual procedure revenue, which is 1,000 patients a year at $20k, adds about $2.69 a share to the base value when it starts in FY2030E [C43]. We ran it through Finagle rather than by hand. Figure 13 shows the result and how it scales. This is a yardstick for partner programs that are not in the forecast (Appendix D). It does not change the base, bull or bear values.

Figure 13 – Value per share added by a flat revenue add-on (base case, Finagle)

Add-on	Patients a year at $20k	Annual revenue (US$M)	Annual EBITDA at 60% (US$M)	Starts	Value per share (US$)	Added per share (US$)
None (base)	–	–	–	–	54.29	–
$20M a year	1,000	20.0	12.0	FY2030E	56.98	2.69
$20M a year	1,000	20.0	12.0	FY2032E	56.57	2.29
$200M a year	10,000	200.0	120.0	FY2030E	81.36	27.07

Source: Finagle reruns of the base case, Oct 25, 2026 [C43][S48].

How the add-on is represented. The add-on is added to revenue in each explicit year from its start year and earns EBITDA at 60%: the 70% procedure gross margin less the 10% clinical-specialist cost [S23][S24][S48] ★. We add no fixed field cost, on the view that a further therapy would use the existing network; the $2M a year per therapy in the forecast would cut the result by about a sixth (derived) [S25]. The FY2032E margin is recomputed, and gsnext is lowered so the FY2033E dollar increment is unchanged, which keeps the add-on flat into FY2033E [C43]. After that, Finagle grows total revenue along its fade, so the add-on rises to about $21M by FY2040E and $25M by FY2045E. Finagle's terminal value then grows it at the 4.95% default. The result is therefore close to, but slightly above, a strictly flat perpetuity. Tax, capex, the NOL and the 10.45% cost of equity are as in the base model.

Scaling. The effect is close to linear: 10,000 patients a year adds $27.07, vs. 10 × $2.69 = $26.93 (derived). Each $1M of annual EBITDA from FY2030E is worth about $0.22 a share (derived). A two-year delay in the start cuts the value by about 15% [C43]. The same yardstick sizes the platform-share risk: each 1,000 Parkinson's procedures a year taken by a competing platform from FY2030E costs about $2.69 a share [S63].

Caveats. (1) There is no dilution or funding: cash goes negative in every case (bear as low as −$157.4M), and an equity raise near $9.25 would cut per-share value. (2) Both approvals are assumed [P1], with no probability weighting. (3) Base and bull PD volumes follow class demand from FY2031E, so they rely on ClearPoint and its partners staffing about 74 centres for Parkinson's alone (95 in the bull) alongside AMT-130, against management's 20–30 [S43][G7]; the bull also needs an 8% treated share [S59]. Every case assumes ClearPoint delivers 100% of one-time PD procedures [S63]; the base and bull assume patients who have had DBS remain eligible [S44] and that class demand, at a treated share about 2.7 times today's DBS rate, is reached [S45]. (4) Value depends heavily on the terminal value and on judgment inputs (mc, gsnext, the steady states and the CAL path). The 20-

year fade assumes growth above the terminal rate for two decades. (5) CAL revenue is booked per study and will be lumpy, and part of it may already be in consensus for FY2027E [G10][S37]. (6) Revenue participation on the Oberland/TPC notes from 2027 is not modelled [D2]. (7) SBC is excluded from EBITDA. We draw no rating from these values.

Appendix A – Bull and Bear Cases

Figure 6 shows the three cases. Price is fixed at $20k throughout [P2]. The cases vary approval timing, ramp speed, utilisation, core growth and costs.

Figure 6 – Scenarios

	Bear	Base	Bull
AMT-130 FY2027E–32E	0 / 270 / 1,000 / 1,800 / 2,300 / 2,700	330 / 2,500 / 3,170 / 3,000 / 3,000 / 3,000	880 / 4,500 / 4,120 / 3,500 / 3,500 / 3,500
One-time PD therapies (class) FY2027E–32E	0 / 0 / 0 / 330 / 2,500 / 4,500	0 / 0 / 3,919 / 7,594 / 11,126 / 10,945	0 / 0 / 5,225 / 9,917 / 14,272 / 13,833
FY2033E AMT-130 / PD class volume in gsnext (share of PD incidence)	3,000 / 4,500 (5.0%)	3,000 / 10,837 (12.0%)	3,500 / 13,530 (15.0%)
One-time PD class demand FY2032E / FY2033E	8,389 / 8,306	10,945 / 10,837	13,833 / 13,530
Eligible pool FY2026E / patients who have had DBS	137,813 / excluded	179,813 / included	179,813 / included
PD treated share of pool (DBS not deducted)	6%	6%	8%
PD volume rule	Capacity ramp to 4,500; capacity binds throughout	Three-year ramp to class demand	Three-year ramp to class demand
ClearPoint share of one-time PD procedures	100%	100%	100%
First PD class approval (bemdaneprocel)	2030	H2 2029	H2 2029
CAL uplift FY2027E–32E (US$M)	0.0 / 2.0 / 4.0 / 6.0 / 8.0 / 10.0	0.0 / 7.0 / 14.0 / 22.0 / 29.0 / 36.0	3.0 / 10.0 / 18.0 / 27.0 / 36.0 / 45.0
Core FY2026E / FY2027E (consensus)	48.0 / 56.0	49.0 / 56.6	50.0 / 57.1
Core growth FY2028E–32E	8%	15%	18%
Core opex growth FY2027E / after	10% / 7%	6% / 5%	6% / 5%
gsnext (FY2033E)	6.2%	5.1%	5.1%
Revenue FY2026E	48.0	49.0	50.0
Revenue FY2027E	56.0	63.1	77.7
Revenue FY2028E	67.9	122.0	167.4
Revenue FY2029E	89.3	230.6	284.4
Revenue FY2030E	119.1	319.9	389.2
Revenue FY2031E	180.2	410.4	502.1
Revenue FY2032E	236.3	428.6	522.3
EBITDA FY2026E	−24.7	−23.6	−22.9
EBITDA FY2027E	−25.2	−19.5	−11.3
EBITDA FY2028E	−24.5	12.0	38.8
EBITDA FY2029E	−16.4	71.2	103.0
EBITDA FY2030E	−5.5	120.6	161.5
EBITDA FY2031E	25.8	170.8	225.0
EBITDA FY2032E	53.7	177.5	232.6
Finagle value per share (US$)	16.72	54.29	72.68

Source: E1, P2, S7, S14–S26, S30–S32, S37, S38, S43–S45, S59, S62–S64, C14–C16, C40, C46, C55.

In the **bull** case, AMT-130 hits the capacity cap in FY2028E and clears the pool in FY2029E. It then settles at 3,500 a year (86% of US incidence). The Parkinson's class launches in FY2029E and follows the base's three-year ramp, but 8% of the pool is treated each year, so volume is 5,225 / 9,917 / 14,272 / 13,833 in FY2029E–32E [S21][S59][C46] ★. That needs

about 95 centres at 3 cases a week by FY2031E, an aggressive build. Core grows 18% and CAL reaches 75% of capacity. The **bear** case is a slow launch in line with gene-therapy precedents [D22][D23]: no AMT-130 revenue in FY2027E, then 270, 1,000, 1,800, 2,300 and 2,700 procedures, reaching 3,000 in FY2033E. The first Parkinson's class approval slips to 2030, a year after the base, and class volume follows a capacity ramp (330 / 2,500 / 4,500 in FY2030E–32E) up to the bear capacity of 4,500 [S22][S43]. The bear also applies the trial-style exclusion of patients who have had DBS, so the pool is about 138,000 and class demand about 8,300–8,400 a year from FY2032E; capacity, not demand, binds in every year, including FY2033E [C55]. CAL reaches only $10M, and core costs grow faster. Bear EBITDA turns positive in FY2031E [S22][S37].

Appendix B – Numbered sources

Documents

- **D1** ClearPoint Neuro, Form 10-K for FY2025 (revenue by line, concentration, IRRAS, NOL), filed Mar 2026: https://www.sec.gov/Archives/edgar/data/1285550/000119312526111231/clpt-20251231.htm (HTML; no page numbers)
- **D2** ClearPoint Neuro, Form 10-Q for Q2 2026 (H1 lines, cash, notes, share count), filed Aug 2026: https://www.sec.gov/Archives/edgar/data/1285550/000119312526330842/clpt-20260630.htm (HTML; no page numbers)
- **D3** ClearPoint Neuro, Q2 2026 results release (Ex. 99.1), Aug 3, 2026: https://www.sec.gov/Archives/edgar/data/1285550/000119312526330541/clpt-ex99_1.htm (HTML)
- **D4** ClearPoint Neuro, FY2025 results release (Ex. 99.1), Mar 17, 2026: https://www.sec.gov/Archives/edgar/data/1285550/000119312526110780/clpt-ex99_1.htm (HTML)
- **D5** ClearPoint Neuro, FY2024 results release with 2025 guidance (Ex. 99.1), 2025: https://www.sec.gov/Archives/edgar/data/1285550/000095017025027716/clpt-ex99_1.htm (HTML)
- **D6** Seeking Alpha, ClearPoint Neuro Q2 2026 earnings call transcript, Aug 2026: https://seekingalpha.com/article/4929731-clearpoint-neuro-inc-clpt-q2-2026-earnings-call-transcript (HTML; paywalled; no page numbers)
- **D7** Seeking Alpha, ClearPoint Neuro Q4 2025 earnings call transcript, Mar 17, 2026: https://seekingalpha.com/article/4883372-clearpoint-neuro-inc-clpt-q4-2025-earnings-call-transcript (HTML; paywalled; read via a transcript copy)
- **D8** Equibles, CLPT analyst estimates, captured Sep 30, 2026: https://equibles.com/stocks/clpt/analyst-estimates (HTML)
- **D9** uniQure, Form 8-K (AMT-130 BLA submission), Sep 2, 2026: https://www.sec.gov/Archives/edgar/data/1590560/000110465926104463/qure-20260902x8k.htm; BioPharma Dive coverage of the action date: https://www.biopharmadive.com/news/uniqure-submit-fda-approval-huntingtons-gene-therapy/829400/ (HTML)
- **D10** ClinicalTrials.gov, NCT06944522 (exPDite-2): https://clinicaltrials.gov/study/NCT06944522 (HTML)
- **D11** ClearPoint Neuro, "Congratulates BlueRock Therapeutics on First Patient Dosed with DA01...", Jun 8, 2021: https://ir.clearpointneuro.com/news-events/press-releases/detail/1040/clearpoint-neuro-inc-congratulates-bluerock-therapeutics (HTML)
- **D12** Tabar et al., Nature, "Phase I trial of hES cell-derived dopaminergic neurons for Parkinson's disease", 2025: https://www.nature.com/articles/s41586-025-08845-y (HTML)
- **D13** MRI Interventions, Form 8-K (Burnett appointment and biography), Oct 10, 2017: https://www.sec.gov/Archives/edgar/data/1285550/000153442417000429/f17-1202.htm (HTML)
- **D14** MRI Interventions, Form 10-K for FY2018 (2017 revenue), filed Apr 1, 2019, accession 0001534424-19-000092: https://www.sec.gov/Archives/edgar/data/1285550/000153442419000092/ (filing index; document not opened)
- **D15** Stockanalysis.com, CLPT analyst ratings, viewed Oct 2026: https://stockanalysis.com/stocks/clpt/forecast/ (HTML)
- **D17** uniQure, AMT-130 Pivotal Update (commercial slides), Sept 2025: https://cdn.craft.cloud/fe68e1a2-b284-4f69-ba29-921f065edf17/assets/uploads/AMT-130-Pivotal-Sept-2025_final_09.24.25.pdf (PDF, p.25)
- **D18** Bruzelius et al., "Huntington's Disease in the United States: Variation by demographic and socioeconomic factors", 2019: https://pmc.ncbi.nlm.nih.gov/articles/PMC6579693/ (HTML)
- **D19** Reuters, "Parkinson's disease at forefront of Bayer bet on cell and gene therapies", Sep 25, 2025: https://www.reuters.com/business/healthcare-pharmaceuticals/parkinsons-disease-forefront-bayer-bet-cell-gene-therapies-2025-09-25/ (paywalled; read via search snippet only)
- **D20** Parkinson's Foundation, Statistics: https://www.parkinson.org/understanding-parkinsons/statistics (HTML)
- **D21** Neurology (2021 abstract 4377), "Incidence and Prevalence of Motor Fluctuations and Deep Brain Stimulation in Parkinson's disease": https://www.neurology.org/doi/10.1212/WNL.96.15_supplement.4377 (HTML)

- **D22** ASGCT 2026 abstract via Larvol, reflections on approved gene therapies: https://delta.larvol.com/Products/?ProductId=d32f5525-efb7-4110-a0a8-4eab2ca91fe6 (secondary source)
- **D23** BioPharma Dive, "BioMarin preaches patience amid slow sales for hemophilia gene therapy": https://www.biopharmadive.com/news/biomarin-roctavian-sales-hemophilia-gene-therapy/708321/ (HTML)
- **D27** Clinical survey abstract, "Reasons for underutilisation of Deep Brain Stimulation (DBS) for Parkinson's Disease (PD): a clinical survey", German Society of Neurosurgery 2012 (GMS): https://doi.org/10.3205/12dgnc047 (HTML; abstract only)
- **D29** "How to improve patient education on deep brain stimulation in Parkinson's disease: the CARE Monitor study", BMC Neurology 2017: https://doi.org/10.1186/s12883-017-0820-7 (HTML)
- **D31** AskBio, "AskBio's Viralgen to Supply Commercial-Scale Drug Product for REGENERATE-PD...", Apr 20, 2026: https://www.askbio.com/viralgen-to-supply-commercial-scale-drug-product-for-regenerate-pd-investigational-gene-therapy-trial-parkinsons-disease/ (HTML)
- **D32** ClinicalTrials.gov, NCT06285643 (REGENERATE-PD, AAV2-GDNF): https://clinicaltrials.gov/study/NCT06285643 (HTML)
- **D33** Marras et al., "Prevalence of Parkinson's disease across North America", npj Parkinson's Disease, 2018: https://doi.org/10.1038/s41531-018-0058-0 (HTML)
- **D34** Macleod et al., "Mortality in Parkinson's disease: a systematic review and meta-analysis", Movement Disorders, 2014: https://doi.org/10.1002/mds.25898 (HTML; abstract)
- **D35** Aarsland et al., "A systematic review of prevalence studies of dementia in Parkinson's disease", Movement Disorders, 2005: https://doi.org/10.1002/mds.20527 (HTML; abstract)
- **D36** "Motor complications in Parkinson's disease: results from 3,343 patients followed for up to 12 years": https://pmc.ncbi.nlm.nih.gov/articles/PMC11145112/ (HTML)
- **D37** CamPaIGN cohort, motor complications at 13-year follow-up: https://pmc.ncbi.nlm.nih.gov/articles/PMC7063985/ (HTML)
- **D38** Rizzo et al., "Accuracy of clinical diagnosis of Parkinson disease", Neurology, 2016: https://doi.org/10.1212/WNL.0000000000002350 (HTML; abstract)
- **D39** Aspen Neuroscience, Form S-1, Oct 2026: https://www.sec.gov/Archives/edgar/data/2124080/000119312526419104/na-20261009.htm (HTML; no page numbers)
- **D51** Morgante et al., Parkinsonism and Related Disorders, 2007 (eligibility for DBS in movement-disorder clinics): https://pubmed.ncbi.nlm.nih.gov/17347021/ (abstract)
- **D52** Moes et al. (D-DATS), Parkinsonism and Related Disorders, 2023 (eligibility for device-aided therapies): https://doi.org/10.1016/j.parkreldis.2023.105359 (HTML; abstract)
- **D53** Gibson et al., "Risk of dementia in Parkinson's disease: a systematic review and meta-analysis", Movement Disorders, 2024: https://doi.org/10.1002/mds.29918 (HTML; abstract)
- **D54** Multidisciplinary DBS screening, Movement Disorders Clinical Practice, 2018: https://pmc.ncbi.nlm.nih.gov/articles/PMC6183455/ (HTML)
- **D40** ClearPoint Neuro, Corporate Presentation, 2Q26 earnings call, Aug 3, 2026, slide 23 (partner programs): https://d1io3yog0oux5.cloudfront.net/_e2d8bbba8f3850e103a2f7c8a42a59c6/clearpointneuro/db/77/2924/pdf/ClearPoint+Neuro+Corporate+Presentation+2Q26+Earnings+Call+-+08.03.26.pdf (PDF, slide 23)
- **D41** BlueRock Therapeutics, Fast Track designation for DA01: https://www.bluerocktx.com/bluerock-therapeutics-receives-fda-fast-track-designation-for-da01-in-the-treatment-of-advanced-parkinsons-disease/ ; RMAT designation for bemdaneprocel: https://www.bluerocktx.com/bluerock-therapeutics-receives-fda-regenerative-medicine-advanced-therapy-designation-for-parkinsons-disease-cell-therapy-candidate-bemdaneprocel/ (HTML)
- **D42** AskBio, Fast Track for AB-1005, Jul 11, 2024: https://www.askbio.com/askbio-receives-fda-fast-track-and-mhra-innovation-passport-designations-for-ab-1005-investigational-gdnf-gene-therapy-for-parkinsons-disease/ ; RMAT, Feb 19, 2025: https://www.askbio.com/askbio-receives-fda-regenerative-medicine-advanced-therapy-designation-for-parkinsons-disease-investigational-gene-therapy/ (HTML)
- **D43** Kenai Therapeutics, press release, Jul 7, 2026 (enrolment complete; Fast Track): https://www.kenaitx.com/press-release-7-7-26 (HTML)
- **D44** Aspen Neuroscience, Fast Track for ANPD001, Oct 19, 2023: https://aspenneuroscience.com/fda-grants-fast-track-designation-to-anpd001-autologous-investigational-cell-therapy-for-the-treatment-of-parkinsons-disease/ ; RMAT, Jul 30, 2026: https://aspenneuroscience.com/aspen-neuroscience-receives-fda-regenerative-medicine-advanced-therapy-rmat-designation-for-sasineprocel-anpd001-to-treat-parkinsons-disease/ (HTML)
- **D45** Neurona Therapeutics, RMAT, Jun 18, 2024: https://www.neuronatherapeutics.com/news/press-releases/061824/ ; ClinicalTrials.gov NCT05135091 (EPIC Phase 3, primary completion Feb 2029): https://clinicaltrials.gov/study/NCT05135091 (HTML)
- **D46** AviadoBio, IND clearance and Fast Track for AVB-101, Nov 6, 2023: https://aviadobio.com/aviadobio-announces-fda-ind-clearance-and-fast-track-designation-for-avb-101-for-the-treatment-of-frontotemporal-dementia-with-progranulin-grn-mutations/ ; ClinicalTrials.gov NCT06064890 (ASPIRE-FTD, primary completion Apr 2027): https://clinicaltrials.gov/study/NCT06064890 (HTML)
- **D47** uniQure, RMAT for AMT-130: https://uniqure.gcs-web.com/news-releases/news-release-details/uniqure-receives-fda-regenerative-medicine-advanced-therapy-rmat ; Breakthrough Therapy: https://uniqure.gcs-web.com/news-releases/news-release-details/uniqure-announces-fda-breakthrough-therapy-designation-granted (HTML)

- **D48** ClinicalTrials.gov NCT07826949 (SRN-101 before resection in recurrent high-grade glioma; not yet recruiting): https://clinicaltrials.gov/study/NCT07826949 (HTML)
- **D49** FDA, approval of KEBILIDI for AADC deficiency, Nov 13, 2024: https://www.fda.gov/news-events/press-announcements/fda-approves-first-gene-therapy-treatment-aromatic-l-amino-acid-decarboxylase-deficiency (HTML)
- **D50** REGENXBIO, Fast Track for RGX-121: https://ir.regenxbio.com/news-releases/news-release-details/regenxbio-receives-fda-fast-track-designation-rgx-121-gene-0/ ; RMAT: https://ir.regenxbio.com/news-releases/news-release-details/regenxbio-receives-fda-regenerative-medicine-advanced-therapy/ ; regulatory update (complete response letter): https://ir.regenxbio.com/news-releases/news-release-details/regenxbio-announces-regulatory-update-rgx-121-bla-mps-ii ; Form 8-K, Aug 24, 2026: https://www.sec.gov/Archives/edgar/data/1590877/000119312526363786/rgnx-20260824.htm (HTML)
- **D55** Asadi-Pooya et al., World Neurosurgery, 2017 (drug-resistant mesial temporal lobe epilepsy prevalence and incidence): https://doi.org/10.1016/j.wneu.2016.12.074 (abstract)
- **D56** Knopman and Roberts, 2011 (frontotemporal lobar degeneration incidence and prevalence): https://pmc.ncbi.nlm.nih.gov/articles/PMC3208074/ (HTML)
- **D57** GeneReviews, GRN frontotemporal dementia: https://www.ncbi.nlm.nih.gov/books/NBK1371/ ; Practical Neurology on ASPIRE-FTD (age 30–75): https://practicalneurology.com/news/fda-fast-track-designation-and-ind-clearance-granted-to-novel-gene-therapy-for-grn-related-frontotemporal-dementia/2470333/ (HTML)
- **D58** AADC deficiency birth prevalence: https://doi.org/10.1002/cns3.20010 ; screening-based range: https://doi.org/10.1016/j.ymgme.2024.108148 (HTML)
- **D59** CDC NVSS, US births 2024 (final): https://www.cdc.gov/nchs/nvss/births.htm (HTML)
- **D60** HRSA, MPS II evidence review, Mar 28, 2022: https://www.hrsa.gov/sites/default/files/hrsa/advisory-committees/heritable-disorders/meetings/mps-ii-final-report-3-28-2022.pdf (PDF)
- **D61** CBTRUS Statistical Report, Neuro-Oncology, 2025: https://academic.oup.com/neuro-oncology/article/27/Supplement_4/iv1/8285946 (HTML)
- **D62** NORD, Friedreich's ataxia: https://rarediseases.org/rare-diseases/friedreichs-ataxia/ (HTML)
- **D63** Brainlab, iPlan Flow 510(k) K053164 ("a dedicated tool for planning trajectories of intra-cranial catheters"): https://www.accessdata.fda.gov/cdrh_docs/pdf5/K053164.pdf ; Cranial Navigation K223288: https://www.accessdata.fda.gov/cdrh_docs/pdf22/K223288.pdf (PDF)
- **D64** FDA GUDID, Brainlab Flexible Catheter 19772 (MR Safe; in commercial distribution; premarket submission K123605, MRI Interventions, SmartFlow catheter): https://fda.report/GUDID/04056481006426 ; K123605: https://www.accessdata.fda.gov/cdrh_docs/pdf12/K123605.pdf (HTML/PDF)
- **D65** CED in diffuse intrinsic pontine glioma, J Neurosurg Pediatr, 2021: https://thejns.org/pediatrics/view/journals/j-neurosurg-pediatr/28/1/article-p34.xml (HTML)
- **D66** Renishaw, neuroinfuse flyer: https://www.renishaw.com/resourcecentre/download/flyer-neuroinfuse-intraparenchymal-drug-delivery-system--100797 ; CDNF extension study release: https://www.renishaw.com/en/45590.aspx (HTML)
- **D67** Whone et al., "Randomized trial of intermittent intraputamenal GDNF in Parkinson's disease", Brain, 2019: https://pmc.ncbi.nlm.nih.gov/articles/PMC6391602/ (HTML)
- **D68** Aspen Neuroscience, ASPIRO uses ClearPoint Navigation System and SmartFlow Cannula, Jun 20, 2024: https://aspenneuroscience.com/parkinsons-disease-clinical-trial-uses-intraoperative-mri-to-guide-precision-implantation-of-patients-own-autologous-replacement-neurons/ (HTML)
- **D69** Neurochase, "Neurochase announces acquisition of strategic Renishaw Neuro Solutions' assets" (agreement announced Nov 20, 2025): https://neurochase.com/Blog/neurochase-announces-acquisition-of-strategic-renishaw-neuro-solutions-assets/ (HTML)
- **D71** ClinicalTrials.gov registry entries, eligibility criteria (API v2, full criteria read Oct 25, 2026): https://clinicaltrials.gov/study/NCT06944522 ; https://clinicaltrials.gov/study/NCT06285643 ; https://clinicaltrials.gov/study/NCT07566429 ; https://clinicaltrials.gov/study/NCT01621581 ; https://clinicaltrials.gov/study/NCT04167540 ; https://clinicaltrials.gov/study/NCT06344026 ; https://clinicaltrials.gov/study/NCT07106021 ; https://clinicaltrials.gov/study/NCT01973543 ; https://clinicaltrials.gov/study/NCT03562494 ; https://clinicaltrials.gov/study/NCT00985517
- **D72** Herzog et al., Mov Disord 2008, DBS in fetal-graft patients (abstract): https://pubmed.ncbi.nlm.nih.gov/18536037/
- **D73** Graff-Radford et al., Arch Neurol 2006, GPi DBS for dyskinesia 11 years after a graft (abstract): https://jamanetwork.com/journals/jamaneurology/fullarticle/791926
- **D74** Freed group, npj Parkinson's Dis 2021, pallidal stimulation in graft recipients (abstract): https://www.nature.com/articles/s41531-021-00183-w
- **D75** MDS abstract, cell-based therapy in patients with an existing DBS system: https://www.mdsabstracts.org/abstract/unilateral-delivery-of-cell-based-therapy-to-the-putamen-and-substantia-nigra-in-patients-with-parkinsons-disease-who-have-an-existing-dbs-system/
- **D76** Mov Disord 2015, "Combining cell transplants or gene therapy with DBS" (abstract): https://doi.org/10.1002/mds.26083
- **D70** Neurochase home page (nFuse: "Currently For investigational use only. Not approved for clinical use in any geography"): https://neurochase.com/ ; ASGCT 2026 poster post ("designed to be compatible with existing neurosurgical robots and frames"): https://www.linkedin.com/posts/fearsomedesign_asgct2026-asgct2026-genetherapy-activity-7460266710869213184-F5YQ (HTML)

- **D16** Finagle discounted-cash-flow valuation runs of the base, bull and bear cases and of the Figure 13 add-on cases, Oct 25, 2026 (model output, not published)
- **D24** Systematic review and meta-analysis of Huntington's disease incidence (regional rates; UK studies incl. Wexler 2016): https://pmc.ncbi.nlm.nih.gov/articles/PMC10086981/ (HTML)
- **D25** Parkinson's Foundation, Prevalence & Incidence: https://www.parkinson.org/understanding-parkinsons/statistics/prevalence-incidence (HTML); Willis et al., npj Parkinson's Disease 2022: https://www.nature.com/articles/s41531-022-00410-y (HTML)
- **D26** Venkatraman et al., J Neurosurg 2024, "Disparities in the treatment of movement disorders using deep brain stimulation" (NIS 2012–19, Table 4): https://pmc.ncbi.nlm.nih.gov/articles/PMC10898494/ (HTML)

Actuals

- **A12** "It is estimated that 10–15% of PD-patients are DBS-candidates". D27.
- **A14** "114 (43.2%) patients preselected as eligible for DBS ... agreed to show up at a specialized DBS center". D29.
- **A15** "By 2030, an estimated 1.2 million people in the U.S. will be living with Parkinson's." D25.
- **A1** Total revenue $31.390M (FY2024A) and $36.971M (FY2025A). D1.
- **A2** FY2024A/FY2025A disaggregated revenue: BDD 17.310/19.040; NNT 10.285/14.831; CES 3.795/3.100. D1, Note 2/MD&A.
- **A3** FY2025A EBITDA (before SBC) −$15.170M = operating loss −24.164 + D&A 0.814 + SBC 8.180. D1 via Finagle's baseline data.
- **A4** H1 2026A revenue $23.008M: BDD 8.839, NNT 11.439, CES 2.730. H1 2025A $17.700M. D2.
- **A5** H1 2026A operating loss −18.722, D&A 0.236, intangible amortization 1.007, SBC 4.489. D3.
- **A6** 2017 revenue $7,379,525. D14.
- **A7** "the cumulative incidence was 1.22 per 100,000 person-years" (age-adjusted, US insured cohort 2003–2016). D18.
- **A8** UK HD incidence 0.67 per 100,000 (Wexler 2016, as compiled in D24); Europe pooled 0.38. D24.
- **A9** "nearly 90,000 people ... diagnosed with Parkinson's disease every year" in the US. D25.
- **A10** DBS for PD in US inpatient discharges: 3,615–4,025 a year 2016–19; 3,990 (1.10% of PD discharges) in 2019. D26, Table 4.
- **A30** At least 930,000 Americans aged 45 or over with PD in 2020 and 1,238,000 in 2030; the authors call these minimum estimates. D33.
- **A31** Survival in PD falls by about 5% for each year of follow-up. D34.
- **A32** Point prevalence of dementia in PD 24.5% across all studies, 31.1% in the highest-quality studies. D35.
- **A33** Off periods affected 35% of patients at 4–6 years and 59% at 8–10 years (3,343 UK patients). D36.
- **A34** Motor fluctuations in 54% of a population-based incident cohort by five years. D37.
- **A35** Pooled accuracy of clinical PD diagnosis against pathology 80.6%. D38.
- **A36** Roughly half of about 300,000 drug-refractory US patients may be eligible for cell-replacement surgery (about 15% of PD). D39.
- **A37** 4.5% of patients eligible for DBS under flexible criteria in Italian movement-disorder clinics. D51.
- **A38** 6.6% eligible for referral to any device-aided therapy in a Dutch general hospital. D52.
- **A39** Dementia incidence in PD 4.45 per 100 person-years. D53.
- **A41** Every PD cell/gene therapy trial checked excludes prior DBS at entry (bemdaneprocel exPDite-2, AB-1005 REGENERATE-PD and Japan, AAV2-GDNF Ph1 and Ph1b, ANPD001 ASPIRO, RNDP-001, VY-AADC01/02, CERE-120); none states a biological or safety reason; RESTORE-1 cites prior surgery that "could complicate the study procedure or negatively impact study evaluations". D71.
- **A42** DBS has been added after cell grafts in small series: 2 patients (D72), 1 patient 11 years after a graft (D73), 3 of 34 graft recipients (D74). Abstracts.
- **A43** Grafts were placed in 3 patients with an existing DBS lead, with adverse events "comparable to standard DBS surgery" (D75); combining cell or gene therapy with DBS is proposed in an expert review (D76). Abstracts.
- **A40** 58.7% of patients evaluated in multidisciplinary DBS screening proceeded to implant. D54. 43.2% of preselected eligible patients agreed to referral to a DBS centre (A14).
- **A21** Drug-resistant MTLE (hippocampal sclerosis): US prevalence 143,000–191,000; incidence 3.1–3.4 per 100,000 a year (× 324M ≈ 10,000–11,000). D55 (abstract).
- **A22** FTLD incidence 2.7–4.1 per 100,000 person-years (age 45–64); US prevalence 20,000–30,000. D56.
- **A23** GRN mutations about 5% of FTD; US GRN-FTD prevalence about 1,000–1,500 (derived from A22 × 5%). D57.

- **A24** ASPIRE-FTD enrols age 30–75. D57 (Practical Neurology).
- **A25** AADC deficiency about 1 in 90,000 births (screening-based range 1/64,000–1/766,660). D58.
- **A26** US births 2024: 3,628,934. D59.
- **A27** MPS II about 1 in 162,000 male births (range 0.26–1.4 per 100,000 births). D60.
- **A28** Glioblastoma: 67,250 US cases 2018–2022, about 13,450 a year. D61.
- **A29** Friedreich's ataxia about 1 in 50,000; about 5,000 US patients. D62.

Guidance and management statements

- **G9** Burnett: prior preclinical capacity "$8 million a year or something like that"; the CAL "could surpass $60 million or so at this facility"; "our revenue miss in the quarter … was largely attributed to a delayed ramp of these preclinical CAL services"; "accelerated adoption of GLP studies at the CAL could go faster than" high-teens 2027 growth; added people are "built into that cost structure". D6.
- **G10** First GLP statement of work "in the multi-million dollar range", expected to be fully recognised by end of H1 2027; 2026 guidance "does not include the execution of any of these large studies"; larger studies expected to start "in the second half of next year". D6; D3.
- **G11** "a single study could be in that $15,000,000 to $20,000,000 range" once GLP capability is in place. D7.
- **G12** Burnett: "if we generally can grow in that 15% to 20%, maybe even north of 20% range for the foreseeable future" (D7); 2027 "high teens, I would say if not 20% growth" with CAL as an accelerator (D6).
- **G1** FY2025 revenue guidance $36.0–41.0M. D5.
- **G2** "adjusting our 2026 revenue guidance to between $48.0 - $52.0 million" (original $52.0–56.0M). Also: 10–15 trials enrolling, about 10 readouts in 18 months, CAL, SONOCARE, commercial reorganization. D3.
- **G3** 2026 guidance "does not include any meaningful expected revenue from commercial drug delivery". D7.
- **G4** "$15,000 to $25,000 per procedure" for a typical uniQure procedure. D6.
- **G5** "$12,000 to $15,000 range per patient" across pharma partners. D6.
- **G6** "operational cash burn to decrease in the second half". D3.
- **G7** 15 centres "could be ready to do 1, if not 2 of these procedures a week"; 20–30 centres would mean "50 to 100 cases a week". D6.
- **G8** "~6,000 patients *Initial Treatable Patients at Launch" (*management estimate); "~20,000 patients Diagnosed HD". D17, p.25.

Estimates

- **E1** Equibles consensus (2 analysts): FY2026E $49.0M (low 48.0, high 50.0); FY2027E $56.55M (low 56.0, high 57.1). D8.
- **E2** Average price target $23.50, 2 analysts (Lake Street $25, Sep 29, 2026; B. Riley $30, Jun 18, 2026; Stifel Buy). D15.

Assumptions

- **P1** Prompt: AMT-130 and bemdaneprocel approved.
- **P2** David (2026-10-23): all procedures $20k, both therapies, all cases; volumes = full use of ClearPoint capacity, ramping, from G7.
- **S1–S3** H2 2026E drivers: BDD = H2 2025A × 1.05; NNT = 2 × Q2 2026A × 1.03; CES = H1 run rate. All scaled to consensus.
- **S4–S6** FY2027E growth 12% / 12% / 5%, scaled to consensus.
- **S7** FY2028E core growth 15% (bear 8%, bull 18%): the low end of management's 15–20% (G12) and FY2027E consensus growth (15.4%, E1).
- **S8** AMT-130 approval at the late Apr/early May 2027 action date (D9); launch Q3 2027.
- **S13** Core gross margin held at H1 2026A 62.9%.
- **S14** H2 2026E core cash opex = H1 × 0.98 (bear × 1.00).
- **S15** FY2027E core cash opex = 2 × H2 × 1.06 (bear 1.10). EBITDA = core gross profit + procedure gross profit − core opex − procedure opex.
- **S16** Core cash opex +5% a year FY2028E–30E (bear +7%).

- **S17** Finagle mc 0.50 (bear 0.45, bull 0.55).
- **S19** Consensus excludes procedure revenue.
- **S20** Base ramp per therapy: launch half-year 10 centres × 1.5/wk × 22 wks ≈ 330; first full year 20 × 2.5 × 50 ≈ 2,500; second full year 30 × 3 × 50 ≈ 4,500; then hold. AMT-130 capped by the pool (G8, C22). First PD class approval (bemdaneprocel) H2 2029.
- **S21** Bull: AMT-130 launch half 20 × 2/wk × 22 ≈ 880, then 4,500 cap; PD class: the base three-year ramp to class demand (S64) on bull class demand (C46): 5,225 / 9,917 / 14,272 / 13,833 FY2029E–32E; FY2033E 13,530.
- **S22** Bear: AMT-130 0 / 270 / 1,000 / 1,800 / 2,300 / 2,700 FY2027E–32E, 3,000 from FY2033E; first PD class approval 2030, the S20 capacity ramp one year later: 330 / 2,500 / 4,500 FY2030E–32E, held at the 4,500 bear capacity from FY2033E (S43); bear class demand (C55) is above capacity in every year. Judgment.
- **S23** Procedure gross margin 70% (10-K: disposables "high margin", no percentage given).
- **S24** Clinical specialist cost 10% of procedure revenue.
- **S25** $2.0M a year of fixed field support per launched therapy.
- **S26** Core growth FY2029E–32E 15% (bear 8%, bull 18%); see S7.
- **S28** US population about 335M (approximation, not sourced).
- **S30** AMT-130 steady state 3,000 a year from FY2030E (David, Oct 23, 2026), as a share of HD incidence (bull 3,500).
- **S31** PD class volume: base and bull, three-year ramp from the FY2029E launch (S64): 1/3, 2/3, then 100% of class demand (C40, C46), class demand from FY2031E; bear, capacity ramp to 4,500 (S22, S43). FY2033E volume in gsnext: base 10,837, bull 13,530, bear 4,500.
- **S32** Explicit window to FY2032E (David, Oct 23, 2026). Core growth as S26; core opex growth as S16. PD class volume: base and bull see S31 and S64; bear see S22.
- **S37** CAL uplift above core growth (US$M, FY2027E–32E): base 0 / 7 / 14 / 22 / 29 / 36 (60% of capacity); bull 3 / 10 / 18 / 27 / 36 / 45; bear 0 / 2 / 4 / 6 / 8 / 10 (G9–G11). Judgment.
- **S43** PD platform capacity: base and bull not capped; volume follows class demand (S64), implying about 74 (base) and 95 (bull) centres at 3 cases/wk × 50 wks for Parkinson's alone by FY2031E, against Burnett's 20–30 centres × 50–100 cases/wk (G7). Bear 4,500, binding from FY2032E. AMT-130 per-therapy cap 4,500 (S20). Aggressive judgment.
- **S44** DBS and one-time therapy are not mutually exclusive: patients who have had DBS stay in the eligible pool, and DBS (3,990 a year, A10) continues alongside and is not deducted from one-time class demand. Trial exclusion of prior DBS is treated as a trial-entry rule (A41–A43). David, Oct 25, 2026: "6% applies to one-time therapy and DBS patients are eligible so I think the pool size should be 180k?" Judgment.
- **S45** From launch in FY2029E, 6% of the opening eligible pool receives a one-time therapy each year (none before). Judgment anchored on the measured DBS rate: about 2.7 times the 2.2% of the 180,000 pool that receives DBS each year (C49), on the view that many more eligible patients would choose a one-time, potentially disease-modifying therapy than symptomatic DBS hardware surgery. No published willingness study supports the multiple. David, Oct 25, 2026: "So we can make it 6% and underpin it on the basis of that 3% (2.9%) receive DBS and we think many more would favour a gene therapy with a disease modifying effect." The 2.9% is 3,990 measured against a pool with prior-DBS patients excluded (137,813; C49); against the 180,000 pool used here the DBS rate is 2.2%. Lifetime uptake 0.06 / (0.095 + 0.06) = 39%, somewhat above a referral × willingness × coverage funnel of about 30% (A14, A40).
- **S46** Half of patients with PD for at least four years and no dementia have motor complications. Judgment anchored to A33 and A34.
- **S47** 90% of otherwise eligible patients are fit for neurosurgery. Judgment; no population figure found.
- **S59** Bull: 8% of the opening eligible pool treated each year from FY2029E (base 6%, S45), on the base three-year ramp; the smallest round change that keeps bull volume above the base in every year. David, Oct 25, 2026, on an earlier bull: "Bull case should show growing to 10k." This bull exceeds 10,000 from FY2030E, to be confirmed by David. Judgment.
- **S62** Class lumping: the PD procedure line is all one-time Parkinson's cell/gene therapy procedures (the class), not one product; volume follows class demand (C40, C46) through the three-year ramp (S64). David, Oct 25, 2026: "And class lumping for the valuation too." Judgment.
- **S63** ClearPoint delivers 100% of one-time Parkinson's cell/gene therapy procedures. David's judgment, Oct 25, 2026: "nobody will risk a multi-billion dollar therapy to save a few thousand on a cannula that wasn't used in the trial". The device used in the pivotal trial becomes part of the approved procedure, so the share stays near 100% for each therapy approved with ClearPoint. Consistent evidence: D12, D68. Devices for other programs not confirmed (D10). Competitor findings: D63–D67, D69, D70. Sizing: about $2.69 a share per 1,000 procedures a year from FY2030E (C43).

- **S64** Three-year ramp to class demand: one-time PD class volume = 1/3 of class demand in FY2029E, 2/3 in FY2030E and 100% from FY2031E (base and bull). David, Oct 25, 2026: "ramp the patient over 3 years up to the demand" and "since that also gives time for other therapies to get approved." The other quantified PD therapies are expected in FY2033E–34E (Appendix D), after the ramp ends, so the rationale is a judgment, not a timeline match. Judgment.
- **S48** Value add-on (Figure 13): flat revenue from the stated start year at 60% EBITDA margin (S23 less S24); no fixed opex; no change to tax, capex or rates. Judgment.
- **S52** Non-BlueRock PD demand split AB-1005 52% / ANPD001 24% / RNDP-001 24% (Appendix D). Judgment.
- **S53–S57** Funnel rates for NRTX-1001, AVB-101, KEBILIDI, RGX-121 and SRN-101 in Figure D2. Judgment.
- **S58** Approval years and ramps for partner programs in Figures D1 and D3: large indications ≈300 → ~1,200 → ~2,200 → steady; one-year ramp for ultra-rare. Judgment.
- **S38** CAL incremental opex 20% of CAL revenue (lease and staff already in core opex, G9); CAL gross margin = core 62.9% (S13). Judgment.
- **S29** Finagle filled inputs: 20-year horizon (Finagle accepts any whole number ≥1; Finagle fades gsnext linearly to its default gt at year 20); cost of debt 8.25% (note minimum coupon, D2; effective rate 11.3%); tax 21%; non-operating assets 0; working-capital change 0; no distributions; debt held flat. Terminal growth left at the Finagle default.

Calculations

- **C1** FY2025A / FY2024A − 1 (A1, A2).
- **C2** H1 2026A EBITDA −12.990 = A5 sum.
- **C3** H2 2026E = E1 − A4 = 25.992.
- **C4** Consensus H2 / unscaled H2 build = 25.992 / 24.26 ≈ 1.07.
- **C5** FY2026E line = A4 + H2E.
- **C6** FY2027E core consensus / unscaled build = 56.55 / 54.48 ≈ 1.04.
- **C7** Total = core (E1, S7, S26) + CAL uplift (S37) + procedure line (P2): 63.150 / 122.033 / 230.567 / 319.886 / 410.426 / 428.642 (FY2027E–32E).
- **C8** Gross margin: 22.692 / 36.971 = 61.4% (FY2025A); H1 2026A 62.9% (D3).
- **C9** FY2026E EBITDA = H1 + H2 revenue × GM − H2 opex = −23.555.
- **C10** EBITDA / revenue.
- **C11** Growth ratios of consecutive EBITDA levels (negative base).
- **C12** FY2032E EBITDA / revenue = 0.414 (bear 0.227, bull 0.445).
- **C13** (36.971 − 36.0) / (41.0 − 36.0) = 0.19 (G1, A1).
- **C14** Finagle base: value per share $54.29 (54.287); equity $1,610.0M; firm $1,659.1M; re 10.45%, gt 4.95% (defaults); min cash −$41.2M; flags "negative cash", "negative FCFE after year 1". D16.
- **C15** Finagle bull: $72.68 (72.682); equity $2,171.1M; min cash −$27.9M. D16.
- **C16** Finagle bear: $16.72 (16.721); equity $464.1M; min cash −$157.4M. D16.
- **C17** 54.29 / 9.25 − 1 = 486.9%; 72.68 / 9.25 − 1 = 685.7%; 16.72 / 9.25 − 1 = 80.8%. Price from Finagle's baseline data (Yahoo previous close, Oct 23, 2026).
- **C18** Line share = line / A1 total.
- **C21** Treatable / diagnosed = 6,000 / 20,000 = 30% (G8).
- **C23** US HD incidence = 335M (S28) × 1.22/100k (A7) = 4,087 a year.
- **C24** US + UK = 4,087 + 68M (S28) × 0.67/100k (A8) = 4,543.
- **C25** 3,000 / 4,087 = 73%; 3,000 / 4,543 = 66% (bull 3,500: 86% / 77%).
- **C27** PV split: Finagle per-share dividends FY2026E–45E discounted at re = $27.90 (base); terminal = $54.29 − $27.90 = $26.39.
- **C28** Sensitivities: Finagle reruns with re ±1 pt, mc ±0.10, gsnext ±5 pts, and year = 10, one at a time; terminal growth unchanged.
- **C29** gsnext = (FY2032E core × core growth S26 + FY2032E CAL increment S37 + FY2033E procedure change × $20k) ÷ FY2032E revenue: base (113.7 × 15% + 7.0 − 108 × $20k) / 428.6 = 5.1%; bull (130.6 × 18% + 9.0 − 303 × $20k) / 522.3 = 5.1%; bear (82.3 × 8% + 2.0 + 300 AMT-130 × $20k) / 236.3 = 6.2%.
- **C33** Steady state / 90,000 (A9): 4,500 = 5.0%; 10,837 = 12.0%; 13,530 = 15.0%.
- **C34** Revenue growth path = consecutive Finagle revenue: base FY2032E 4.4%, FY2033E 5.1%, FY2045E 5.0%.
- **C35** CAL FY2032E EBITDA = 36.0 × (62.9% − 20%) = 15.4 (S13, S37, S38).

- **C40** One-time PD class demand = one-time treated in the eligible-pool model (6% of the opening pool from FY2029E, S45; DBS not deducted, S44): 11,756 / 11,391 / 11,126 / 10,945 (FY2029E–32E); 10,837 (FY2033E); 10,792 (FY2034E); 10,801 (FY2035E); 11,454 (FY2040E); 12,780 (FY2045E). Growth FY2033E–40E 0.8% a year. Appendix C.
- **C46** Bull class demand = pool model (C45 inputs) re-run with 8% treated from FY2029E (S59): 15,674 / 14,875 / 14,272 / 13,833 (FY2029E–32E); 13,530 (FY2033E); 13,250 (FY2035E); 13,792 (FY2040E). Steady-state treated share of entrants 0.08 / (0.095 + 0.08 + 0.029) = 39%.
- **C48** Value decomposition (base, Finagle reruns of copies): PD volume held at 5,000 capacity after FY2032E with the pool excluding prior-DBS patients (137,813): $39.25 (gsnext 7.8%); same capacity path with the 179,813 pool: $39.25; three-year ramp to class demand (as modelled): $54.29 (gsnext 5.1%).
- **C49** DBS rate = 3,990 DBS a year (A10) ÷ eligible pool: 179,813 at start FY2026E = 2.2%; 190,401 at start FY2028E = 2.1%. 6% ÷ 2.2% = 2.7. Against a pool excluding prior-DBS patients (137,813) the same 3,990 is 2.9%. Pool model.
- **C41** FY2026E US PD prevalence = log-linear interpolation of 930,000 (2020) and 1,238,000 (2030) (A30): growth 2.9% a year, 1.104M in FY2026E.
- **C42** Eligible pool FY2026E = 1.104M (C41) × (0.729 − 0.28) (A31, A32, C44) × 0.50 (S46) × 0.806 (A35) × 0.90 (S47) = 179,813, 16.3% of PD; patients who have had DBS included (S44).
- **C43** Figure 13: Finagle reruns of copies of the base case with revenue +20 (or +200) and EBITDA +12 (or +120) in each explicit year from FY2030E (or FY2032E); sales and EBITDA growth lists and me recomputed; gsnext × FY2032E revenue / (FY2032E revenue + add-on). Base $54.29 → $56.98 (+$2.69); FY2032E start $56.57 (+$2.29); +200 → $81.36 (+$27.07).
- **C44** Share of prevalent patients diagnosed four or more years = exp(−4 × (0.05 + 0.029)) = 0.73, survival loss 5% a year (A31) weighted for 2.9% prevalence growth (C41); cross-check 1.1M prevalent / 90,000 new a year ≈ 12 years lived with diagnosis (A9). Dementia (0.28, midpoint of A32) is subtracted from this group, not multiplied.
- **C45** Leave rate m = 5% death (A31) + 4.45% new dementia (A39) = 0.095 a year; entrants FY2026E = (m + g) × pool = 0.124 × 179,813 = 22,300, growing 2.9% a year; no one-time therapy before launch, so the pool grows at g to FY2028E; pool end = start + entrants − leavers − treated.
- **C50** Partner steady states = Figure D2 funnel results, rounded (PD to 100s; AVB-101 to 5s). **C52** NRTX-1001 = 10,500 × 0.85 × 0.5 × 0.5 × 0.8 × 0.7 = 1,249 + 167,000 (A21 mid) × 11.9% ÷ 10 = 1,987 → 3,200. **C53** FTD-GRN incidence = ~83M US aged 45–64 (unsourced) × 3.4/100k ≈ 2,800 × 5% = 140.
- **C54** Illustrative non-BlueRock PD rows = min(launch ramp, S52 share of class demand C40 less BRT-DA01) each year; FY2040E 2,200 / 1,000 / 1,000 (launch ramps bind in every year). Not used in the valuation, which uses the class (S62).
- **C55** Bear class demand = pool model with patients who have had DBS excluded (3,990 ÷ 0.095 ≈ 42,000; pool 137,813, trial-style, A41), 6% treated: 9,010 / 8,731 / 8,527 / 8,389 (FY2029E–32E); 8,306 (FY2033E). Above the 4,500 bear capacity in every year (S43).
- **C30** Finagle template table: Finagle's annual output for each case, laid out row for row as in Finagle's standard company template.

Not verified

- D7: Q4 2025 call transcript read via a transcript copy; link unverified.
- D14: 2017 revenue taken from a secondary extract of the filing; document not opened, page unverified.
- Burnett's shareholding: not sourced.
- Consensus treatment of approval revenue (S19): unverified.
- No BlueRock–ClearPoint or uniQure–ClearPoint commercial supply agreement was found.
- D19 (Bayer readout 2028–29): Reuters article paywalled; read via search snippet only.
- S23 procedure gross margin: no disposables margin disclosed.
- S28 US (335M) and UK (68M) populations: approximations, unsourced.
- A8 UK incidence taken from the D24 meta-analysis compilation, primary study not opened.
- A10 DBS counts are inpatient (NIS); outpatient DBS is not captured, so the DBS calibration may understate the surgical market.
- D27: abstract only; full text not opened. A12's 10–15% is a widely quoted estimate, not a US measurement, and its criteria are not stated (likewise A36).
- Zolgensma uptake: not sourced, so not used as an analog.
- G9–G11 quotes read from a transcript copy copies of D6/D7 (Seeking Alpha, paywalled).
- Parkinson's pool (Appendix C): motor-complication share (S46), fitness (S47), the 6% treatment rate (S45, about 2.7 times the DBS rate; the multiple is unsupported by any published study), keeping patients who have had DBS in the pool (S44; trials exclude them, A41; the combination evidence is small case series read as abstracts, A42, A43),

the three-year ramp (S64) and the 100% platform share (S63; partly supported: no Brainlab CED cannula found, but Renishaw has an investigational MRI-compatible CED system, D63–D67) are judgments or approximations. Inputs A30–A40 are carried from the pool model's sources and were not re-opened for this report; several (D34, D35, D38, D51–D53) are cited from abstracts.

- C44: the 0.73 share diagnosed four or more years is a model calculation; no source gives it directly.
- Partner programs (Appendix D): designations are not printed on slide 23 (D40) and come from partner releases. Siren SRN-101: no FDA designation found. KEBILIDI: RMAT/ Fast Track not confirmed (Priority Review, Orphan and rare pediatric disease voucher confirmed). Three undisclosed programs (two PD, one Friedreich's ataxia) are not quantified. US population aged 45–64 (~83M) in C53 is not sourced. A21 and A22 were read from abstracts or search snippets; the ASPIRE-FTD age range is from Practical Neurology, not the registry. KEBILIDI procedure volumes are not disclosed.

Appendix C – Parkinson's eligible-pool model

This appendix sets out the model behind Parkinson's demand in the forecast (Section 5, Figure 4b). It works at the level of the class of one-time cell and gene therapies, not any single product, and it is a single central scenario. Each input is either a published statistic or a labelled judgment.

Starting prevalence. We start from the Parkinson's Foundation prevalence project, which estimates at least 930,000 Americans aged 45 or over with PD in 2020 and 1,238,000 in 2030 [A30]. Interpolating log-linearly between these points implies growth of 2.9% a year and about 1.10 million patients in FY2026E [C41]. The authors call their figures minimum estimates, because they rely on diagnosed, recorded cases [A30]. The Foundation quotes the same 1.2 million for 2030 [A15].

Medical eligibility. Eligibility follows the DBS label rather than trial entry criteria: levodopa-responsive PD of at least four years' duration, with motor complications, no dementia and fit for surgery. There is no age cap.

- *Duration.* Survival in PD falls by about 5% for each year of follow-up [A31]. With prevalence growing 2.9% a year, about 73% of prevalent patients were diagnosed four or more years ago [C44]. As a check, about 90,000 new diagnoses a year [A9] against 1.1 million prevalent cases implies about 12 years lived with the diagnosis on average, which gives a similar share [C44].
- *Dementia.* A systematic review puts the point prevalence of dementia in PD at 24.5% across all studies and 31.1% in the highest-quality ones [A32]; we use 28%. Dementia is rare in the first years after diagnosis, so we subtract it from the four-year group rather than multiply. That leaves 45% of all patients [C44].
- *Motor complications.* We assume half of that group have them [S46] ★. In 3,343 UK patients, off periods affected 35% at 4–6 years and 59% at 8–10 years [A33]. A population-based incident cohort found motor fluctuations in 54% by five years [A34].
- *Diagnosis.* About 81% are taken to have true, levodopa-responsive idiopathic PD, the pooled accuracy of clinical diagnosis against pathology [A35]. It is a proxy, not a measured response rate.
- *Fitness.* We assume 90% are fit for neurosurgery [S47] ★. We found no population figure.

Patients who have had DBS. About 3,990 PD patients receive DBS each year [A10]. They stay in the pool. Every one-time-therapy trial we checked excludes prior DBS [A41], but no registry entry gives a biological or safety reason, and DBS has been combined with grafts in both orders in small case series [A42][A43]. We therefore treat the exclusion as a trial-entry rule, not a label rule [S44] ★. The bear applies it: removing the roughly 42,000 living DBS patients (3,990 a year ÷ the 0.095 leave rate) leaves about 138,000 [C55].

Figure C1 shows the funnel from prevalence to the medically eligible pool.

Figure C1 – Eligible-pool funnel, FY2026E

Step	Factor	Patients	Basis
US PD prevalence	–	1,104,000	Published, interpolated [A30][C41]
Diagnosed ≥4 years and no dementia	0.73 − 0.28	496,000	Published, calculated [A31][A32][C44]
Motor complications ★	× 0.50	248,000	Judgment anchored to [A33][A34] [S46]
Levodopa-responsive idiopathic PD	× 0.81	200,000	Published proxy [A35]
Fit for surgery ★ = medically eligible	× 0.90	180,000	Judgment [S47]
Eligible pool (DBS patients included)		**180,000**	[C42][S44]

Source: tags as shown. ★ = judgment call. May not sum due to rounding.

Checks on the eligible share. Medically eligible patients make up 16% of PD (derived) [C42], which agrees with two published figures near 15%. Aspen Neuroscience's filing says roughly half of about 300,000 drug-refractory US patients may be eligible for cell-replacement surgery, about 15% of PD [A36]. A German conference abstract cites 10–15% [A12]. Neither states its criteria, so both confirm a share, not a level. Clinic studies give a lower bound: 4.5% under flexible criteria in Italian movement-disorder clinics [A37] and 6.6% eligible for referral to any device-aided therapy in a Dutch general hospital [A38]. Those studies use stricter clinical criteria, including age and severity, so a label-based pool should sit above them.

Flows. Each year, the closing pool equals the opening pool plus entrants, less leavers and patients treated [C45].

- *Leavers.* A share of the pool leaves each year through death (5% a year) [A31] or new dementia (4.45 per 100 person-years) [A39], 9.5% in total, so patients stay eligible for about ten years on average [C45].
- *Entrants.* First-year entrants are (m + g) × the starting pool, about 22,300 in FY2026E, so that with no one-time therapy the pool grows at the same 2.9% as prevalence to FY2028E; entrants then grow 2.9% a year [C45].
- *Pool growth.* Entrants exceed leavers, so the pool grows unless treatment absorbs the gap.
- *Treated.* No one receives a one-time therapy before launch in FY2029E. From launch, 6% of the opening pool is treated each year [S45] ★, about 2.7 times the 2.2% of the pool that receives DBS each year [C49]. We judge that many more eligible patients would choose a one-time, potentially disease-modifying therapy than symptomatic hardware surgery; no published willingness study measures this. DBS continues alongside at 3,990 a year and is shown for context only: it is not deducted from one-time demand, and patients who have had DBS remain eligible [S44].

Figure C2 shows that one-time treatment jumps at launch as the pre-launch backlog is drawn down, then eases and grows with entrants.

Figure C2 – Pool model results

FY	2026E	2029E	2030E	2032E	2035E	2040E	2045E
Eligible pool, start of year (000s)	180	196	190	182	180	191	213
Entrants (000s)	22	24	25	26	29	33	38
One-time therapy class demand (6% of pool, 000s)	–	11.8	11.4	10.9	10.8	11.5	12.8
Class demand ÷ entrants	–	0.48	0.46	0.41	0.37	0.34	0.33
Memo: DBS (000s; not deducted)	4.0	4.0	4.0	4.0	4.0	4.0	4.0

Source: A10, A30–A35, A39, S44–S47, C40–C45.

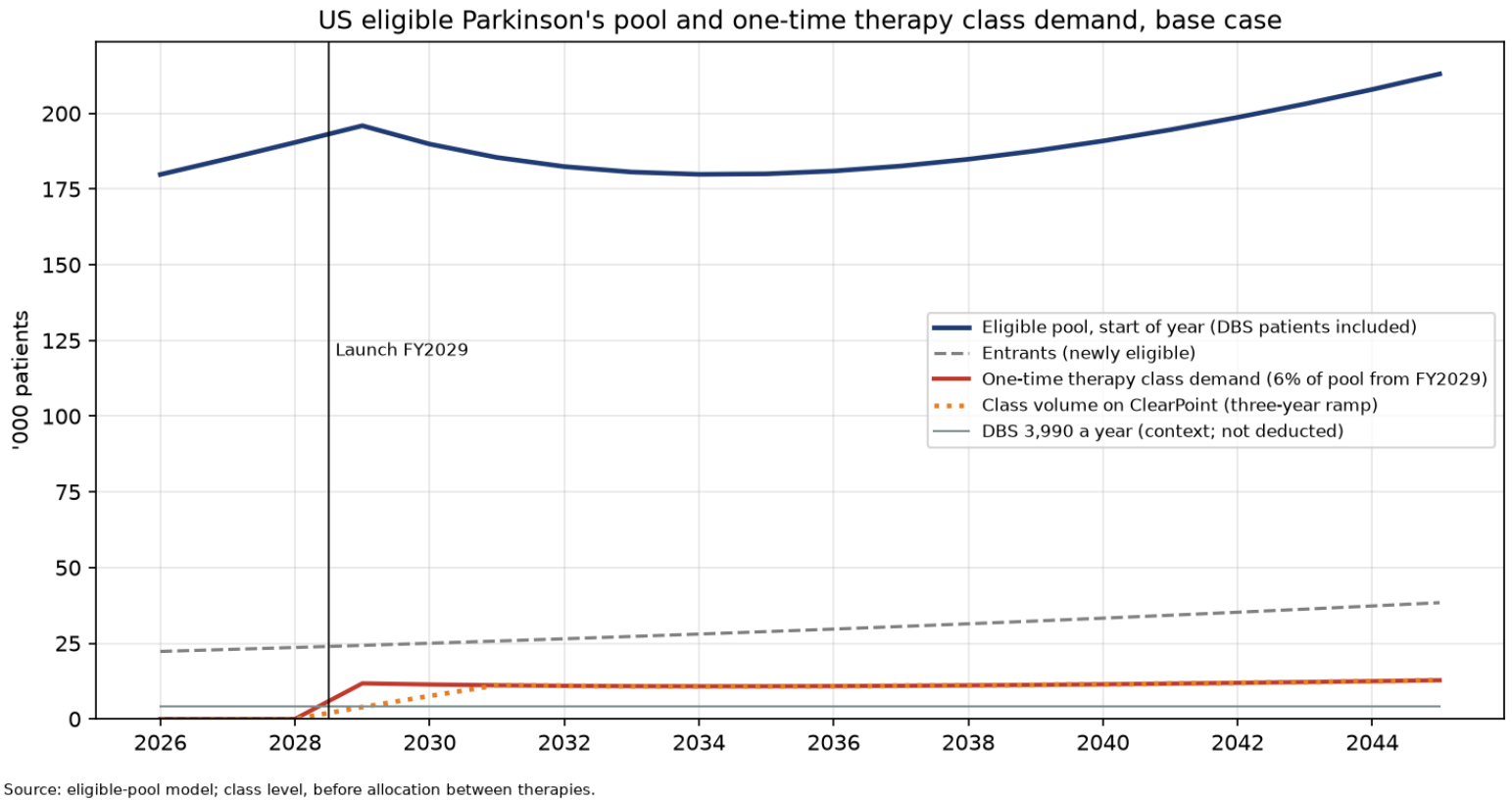


Figure C3 – Eligible pool, one-time therapy class demand and class volume (three-year ramp), FY2026E–FY2045E, base case. DBS (3,990 a year) is shown for context and is not deducted. Source: C40–C45.

Reading the results. One-time treatment starts at about 11,800 in the launch year; the high first year comes from the backlog built up before launch [C40]. The pool then shrinks by about 8% over five years and grows again with entrants. Class demand settles at a constant share of entrants, 0.06 / (0.095 + 0.06 + 0.029) ≈ 33% (derived) [C45]. That is a different quantity from the share of any single group of entrants who are eventually treated, 0.06 / (0.095 + 0.06) ≈ 39% (derived), because each year's entrants outnumber the previous year's. There is no second wave. As a plausibility check, a judgment funnel of referral (55%) × willingness (60%) × coverage (90%) gives about 30% treated [S45]; willingness is anchored to published consent and implant rates [A14][A40]. A 6% treatment rate therefore implies somewhat more uptake than that funnel. At $20,000 a patient, FY2040E class demand is worth about $229M (derived) [C40][P2].

Weak points. (1) The motor-complication share and medical fitness are judgments, and together they move the pool more than any published input [S46][S47]. (2) The diagnosis factor is a proxy for levodopa response [A35]. (3) The dementia midpoint, and the assumption that dementia sits almost entirely in patients diagnosed four or more years ago, are simplifications [A32]. (4) The 5% death rate comes from all PD patients; patients without dementia probably die less often, which would lengthen time in the pool [A31]. (5) The pool includes patients who have had DBS, on the assumption that they are eligible for one-time therapy [S44]; every trial so far excludes them [A41], and if labels did too the pool would be about 138,000, as in the bear [C55]. (6) The 3,990 counts inpatient procedures only [A10]. (7) Prevalence is a published minimum, so the pool is more likely too low than too high [A30]. (8) The 6% treatment rate and the FY2029E launch are assumptions [S45]. The 6% rests on the judgment that a one-time therapy would draw about 2.7 times DBS's take-up, which no published willingness study supports [C49]. The often-quoted 2.9% DBS rate is measured against a pool that excludes prior-DBS patients (138,000), not the 180,000 used

here [C49]. The bull uses 8% (Figure 4c) [S59]. The model treats the whole class as launching in FY2029E, while other one-time therapies are likely to launch from FY2033E (Appendix D). (9) DBS and one-time therapy are modelled independently; a patient who has both (for example DBS after a graft for residual symptoms) is not counted as an extra ClearPoint procedure, which is on the low side [A42].

Appendix D – Partner programs

ClearPoint lists more than ten partner programs under FDA expedited review [D40]. **Two lines are in our forecast and valuation: the class of one-time Parkinson's cell/gene therapy procedures (Section 5), of which BRT-DA01 (bemdaneprocel) is the first to launch, and AMT-130 (Huntington's).** Because the valuation counts the whole Parkinson's class, the split of that class among the four Parkinson's therapies below (BRT-DA01, AB-1005, ANPD001, RNDP-001) is illustrative only and adds nothing to it. The non-Parkinson's programs other than AMT-130 are not in the forecast or valuation. We show them to size the optionality; Figure 13 gives a per-share yardstick of about $2.69 for each 1,000 patients a year at $20k [C43]. All volumes are US, at $20k per procedure [P2], and base cases are central estimates.

Figure D1 shows that the quantified programs would reach about 16,000 procedures and $320M of revenue a year by FY2040E if all were approved. Of these, AMT-130 (3,000) is in the forecast, and the Parkinson's class is in the forecast at class demand (11,454 in FY2040E) rather than as the sum of the four Parkinson's rows (9,200).

Figure D1 – Partners, incidence, steady-state patients and revenue

Partner	Therapy	Indication	FDA designation(s)	US incidence / yr	Steady-state patients / yr (base)	Revenue / yr at $20k (US$M)	Plausible approval
BlueRock (Bayer)	BRT-DA01 (bemdaneprocel), first of the PD class	Parkinson's disease	Fast Track (2021) [D41]; RMAT (May 2024) [D41]	~90,000 [A9]; prevalence 1.2M by 2030 [A15]	5,000 [C54] (FY2040)	100.0	H2 2029 [S20]
AskBio (Bayer)	AB-1005 (AAV2-GDNF)	Moderate Parkinson's disease	Fast Track (Jul 2024) [D42]; RMAT (Feb 2025) [D42]	~90,000 (shared PD pool) [A9]	2,200 [C54] (FY2040)	44.0	2033 ★ (Ph2 completes Aug 2028 [D32])
Kenai	RNDP-001	Moderate to moderate-severe idiopathic PD	Fast Track [D43]	~90,000 (shared PD pool) [A9]	1,000 [C54] (FY2040)	20.0	2034 ★ (Ph1b/2a enrolled Jul 2026 [D43])
Aspen	ANPD001 (sasineprocel)	Parkinson's disease	Fast Track (Oct 2023) [D44]; RMAT (Jul 30, 2026) [D44]	~90,000 (shared PD pool) [A9]	1,000 [C54] (FY2040)	20.0	2034 ★ (Ph1/2a ASPIRO [D44])
Undisclosed	–	Parkinson's disease	Not shown on slide	~90,000 (shared) [A9]	not quantified	–	–
Undisclosed	–	Parkinson's disease	Not shown on slide	~90,000 (shared) [A9]	not quantified	–	–
Neurona	NRTX-1001 (rezanecel)	Drug-resistant mesial temporal lobe epilepsy	RMAT (Jun 2024) [D45]	~10,000–11,000 [A21]; prevalence 143k–191k [A21]	3,200 [C50]	64.0	2030 ★ (Ph3 primary completion Feb 2029 [D45])
AviadoBio	AVB-101	Frontotemporal dementia, GRN mutation	Fast Track (Nov 2023); Orphan [D46]	~140 (derived) [C53]; prevalence ~1,000–1,500 [A23]	40 [C50]	0.8	2031 ★ (Ph1/2 primary completion Apr 2027 [D46])
Undisclosed	–	Friedreich's ataxia	Not shown on slide	~70 births (1/50,000 × 3.63M, derived) [A29][A26]; prevalence ~5,000 [A29]	not quantified	–	–
uniQure	AMT-130	Huntington's disease	RMAT (Jun 2024); Breakthrough (Apr 2025) [D47]	~4,087 [C23]	3,000 [S30]	60.0	2027 (action date late Apr/early May 2027 [D9])
Siren	SRN-101	Recurrent high-grade glioma	None found (preclinical/Ph1 not yet recruiting) [D48] – unverified	~13,450 glioblastoma [A28]	530 [C50]	10.6	2034 ★ (Ph1 not yet recruiting [D48])
PTC	KEBILIDI / Upstaza	AADC deficiency	Approved Nov 13, 2024 (accelerated); Priority Review, Orphan, RPD voucher [D49]; RMAT/Fast Track unverified	~40 (derived) [A25][A26]	14 [C50]	0.3	Approved 2024 [D49]
REGENXBIO	RGX-121 (clemidsogene lanparvovec)	Hunter syndrome (MPS II, neuronopathic)	Fast Track (2018); RMAT (2023); Orphan; RPD [D50]	~11 (derived) [A27][A26]	2 [C50]	<0.1 (0.04)	2029 ★ (CRL Feb 2026; clinical hold Aug 2026 [D50])
Total (quantified)					**15,986**	**319.7**	

Source: slide 23 [D40]; tags as shown. Designations are not on the slide (its title says "10+ Programs Are Already Under FDA Expedited Review"); they come from partner releases. PD therapies share one patient pool (Figure D2a), so their rows are not independent. Steady state = FY2040E. Non-PD rows are flat from FY2036E (epilepsy includes backlog drawdown). The four PD rows together reach 9,200 in FY2040E, below class demand of 11,454, because each later product is limited by its own launch ramp.

The four Parkinson's therapies share one patient pool, so we allocate one-time class demand (Appendix C) among them rather than size each separately. Each year, one-time class demand = 6% of the eligible pool (DBS not deducted). In this illustration BRT-DA01 keeps a single-product launch ramp to 5,000 (330 → 2,500 → 4,500 → 5,000) [S20], capped at class demand; the cap never binds, as class demand is 10,800–11,800 a year to FY2040E. The remainder (class − bemdaneprocel) is split AB-1005 52% / ANPD001 24% / RNDP-001 24% [S52] ★. Each row is the smaller of its launch ramp (AB-1005 300 → 1,200 → 2,200; ANPD001 and RNDP-001 200 → 600 → 1,000) and its share of demand; with the 180,000 pool the

launch ramps bind in every year. Demand not taken up before a product launches is not carried forward ★. FY2040: 5,000 / 2,200 / 1,000 / 1,000. In the forecast and valuation, class volume follows the three-year ramp to class demand and is not split (S62, S64).

Figure D2a – Funnel: PD class (shared) – eligible-pool model

Step	Rate / input	Result	Source
US PD prevalence FY2026 (≥930k in 2020, 1.238M in 2030, log-linear; minimum estimates)	+2.9%/yr	~1,104,000	A30, C41
× diagnosed ≥4 yrs and no dementia (0.73 − 0.28)	45%	~496,000	A31, A32, C44
× motor complications ★	50%	~248,000	S46, A33, A34
× levodopa-responsive idiopathic PD (diagnostic-accuracy proxy)	80.6%	~200,000	A35
× fit for surgery ★ = medically eligible (16% of PD)	90%	~180,000	S47
= eligible pool, start FY2026 (DBS patients included) ★		~180,000 (179,813)	C42, S44
Each year: + entrants (0.124 × pool in FY2026, then +2.9%/yr) − leavers (m = death 5% + dementia 4.45%) − one-time treated			A31, A39, C45
Treated from FY2029 = 6% of pool ★ (about 2.7× DBS's 2.2%; none before) = one-time therapy class demand	6%	FY2029 ~11,800; FY2035 ~10,800; FY2040 ~11,500	S45, C40, C49
Memo: DBS 3,990 a year, context only, not deducted	3,990	–	A10, S44

Source: tags as shown. ★ = judgment call.

Figure D2b – Funnel: NRTX-1001

Step	Rate / input	Result	Source
DR-MTLE incidence (3.1–3.4/100k × 324M)	10,500	10,500	A21
× adults in trial age range ★	85%	8,925	S53
× identified/referred at epilepsy centre ★	50%	4,462	S53
× willing (non-destructive one-time vs resection/LITT) ★	50%	2,231	S53
× access/reimbursement ★	80%	1,785	S53
× share vs resection, LITT, RNS ★	70%	1,250	S53
+ prevalent backlog 143k–191k × same screens (11.9%) ÷ 10 yrs		1,987	A21, C52

Source: tags as shown. ★ = judgment call.

Figure D2c – Funnel: AVB-101

Step	Rate / input	Result	Source
FTD incidence ages 45–64, ~3.4/100k × ~83M	2,800	2,800	A22, C53
× GRN mutation share 5%	5%	140	A23
× genetically diagnosed ★	60%	84.0	S54
× early-stage, age 30–75 per trial ★	70%	58.8	S54, A24
× willing ★	60%	35.3	S54
× access ★	80%	28.2	S54
× share vs other GRN therapies ★	50%	14.1	S54
+ backlog 1,000–1,500 prevalent × same screens ÷ 5 yrs		25.0	A23, C53

Source: tags as shown. ★ = judgment call.

Figure D2d – Funnel: KEBILIDI

Step	Rate / input	Result	Source
US births × 1/90,000	40	40.3	A25, A26
× diagnosed ★	60%	24.2	S55
× eligible per label ★	90%	21.8	S55
× willing ★	80%	17.4	S55
× access ★	80%	13.9	S55
× share (only approved therapy)	100%	13.9	S55

Source: tags as shown. ★ = judgment call.

Figure D2e – Funnel: RGX-121

Step	Rate / input	Result	Source
Male births × 1/162,000	11	11.5	A26, A27
× neuronopathic ~2/3 ★	67%	7.7	S56
× diagnosed young enough ★	70%	5.4	S56
× willing ★	80%	4.3	S56
× access ★	80%	3.4	S56
× share vs ERT/brain-penetrant ERT ★	60%	2.1	S56

Source: tags as shown. ★ = judgment call.

Figure D2f – Funnel: SRN-101

Step	Rate / input	Result	Source
Glioblastoma incidence (CBTRUS)	13,450	13,450	A28
× recurrence ★	90%	12,105	S57
× resectable at recurrence ★	30%	3,632	S57
× willing ★	70%	2,542	S57
× access ★	70%	1,779	S57
× share vs other recurrent-GBM options ★	30%	534	S57

Source: tags as shown. ★ = judgment call.

Figure D3 shows the ramps, assuming every program is approved.

Figure D3 – Ramp, procedures by year (100% approval)

Therapy	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E	FY2036E	FY2037E	FY2038E	FY2039E	FY2040E
BRT-DA01	–	–	330	2,500	4,500	5,000	5,000	5,000	5,000	5,000	5,000	5,000	5,000	5,000
AMT-130	330	2,500	3,170	3,000	3,000	3,000	3,000	3,000	3,000	3,000	3,000	3,000	3,000	3,000
AB-1005	–	–	–	–	–	–	300	1,200	2,200	2,200	2,200	2,200	2,200	2,200
ANPD001	–	–	–	–	–	–	–	200	600	1,000	1,000	1,000	1,000	1,000
RNDP-001	–	–	–	–	–	–	–	200	600	1,000	1,000	1,000	1,000	1,000
NRTX-1001	–	–	–	300	1,200	2,200	3,200	3,200	3,200	3,200	3,200	3,200	3,200	3,200
AVB-101	–	–	–	–	10	25	40	40	40	40	40	40	40	40
KEBILIDI	12	14	14	14	14	14	14	14	14	14	14	14	14	14
RGX-121	–	–	1	2	2	2	2	2	2	2	2	2	2	2
SRN-101	–	–	–	–	–	–	–	100	300	530	530	530	530	530

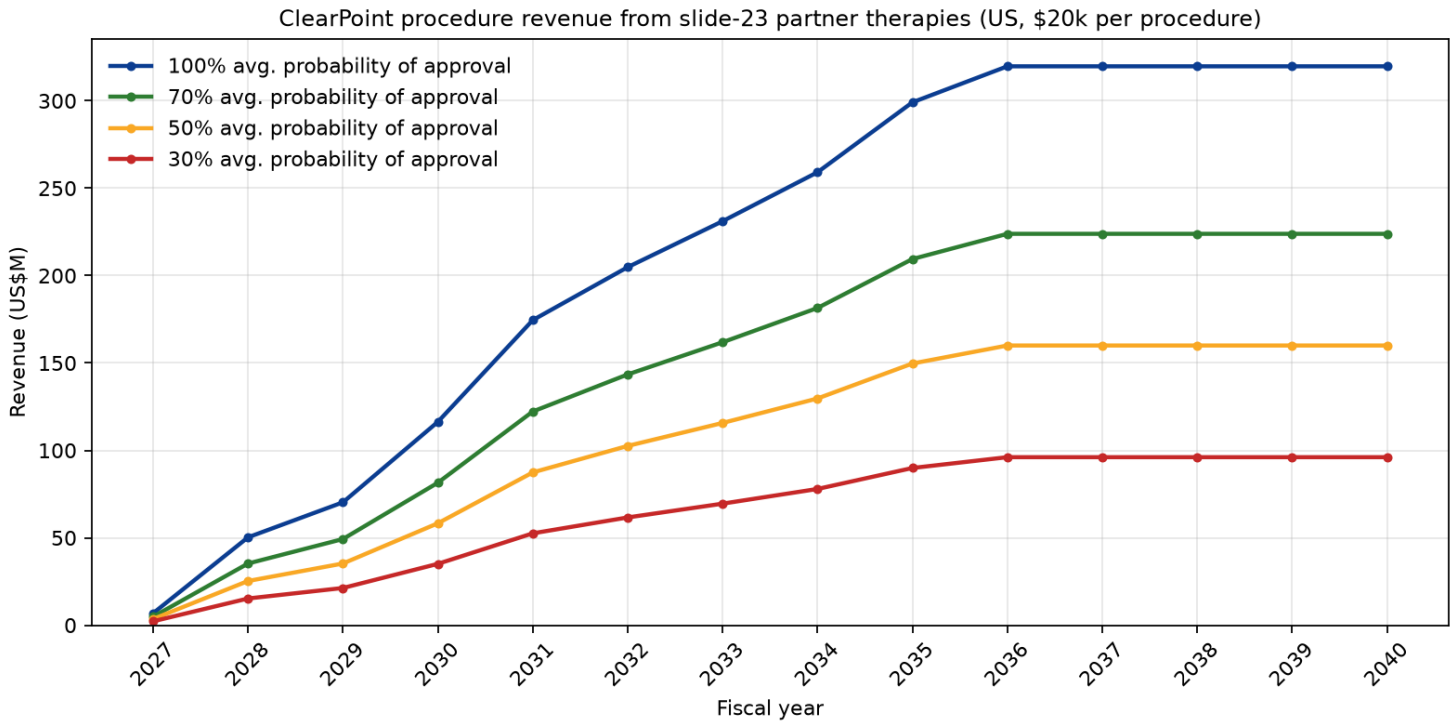
Source: S20, S30, S52, S58, C40, C50, C54. In the forecast: AMT-130 and the four PD rows as one class (S62); the PD split is illustrative.

Figure D4 shows total partner procedure revenue at different average probabilities of approval.

Figure D4 – Total partner procedure revenue (US$M) by average probability of approval

Probability	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E	FY2036E	FY2037E	FY2038E	FY2039E	FY2040E
100%	6.8	50.3	70.3	116.3	174.5	204.8	231.1	259.1	299.1	319.7	319.7	319.7	319.7	319.7
70%	4.9	35.3	49.3	81.5	122.2	143.5	161.9	181.5	209.5	223.9	223.9	223.9	223.9	223.9
50%	3.5	25.3	35.3	58.3	87.4	102.6	115.7	129.7	149.7	160.0	160.0	160.0	160.0	160.0
30%	2.2	15.3	21.3	35.1	52.6	61.6	69.5	77.9	89.9	96.1	96.1	96.1	96.1	96.1

Source: Figure D3 × $20k [P2] × probability. KEBILIDI is already approved and held at 100% in every row. Approval probabilities are applied to each therapy independently, although the four Parkinson's therapies share one pool; if one fails, the others could take more of its demand, so the lower rows probably understate PD revenue.



Source: ClearPoint 2Q26 corporate presentation slide 23 (D40); PD class demand from US eligible-pool model (6% of a 180k pool; DBS not deducted); other funnels S53–S58; price $20k [P2].
KEBILIDI (approved Nov 2024) held at 100%. Undisclosed Parkinson's (×2) and Friedreich's ataxia programs not quantified.
Base-case volumes; scenario, not guidance. PD therapies share one pool but approval is applied independently.

Figure D5 – Partner procedure revenue by probability of approval, FY2027E–FY2040E. Source: Figure D4.

Limitations. Demand that arises before a later Parkinson's product launches is not carried forward, which is simple and on the low side. The 52 / 24 / 24 split of non-BlueRock PD demand is a judgment [S52]. Funnel rates for the other indications are judgments [S53]–[S57]. Three undisclosed programs (two Parkinson's, one Friedreich's ataxia) are not quantified.

Appendix E – Method

We started from FY2025A reported lines and H1 2026A. Because 2026 guidance was revised, totals for FY2026E and the FY2027E core business are set to analyst consensus. Line forecasts come from simple drivers, scaled proportionally to those totals. The core business then grows at assumed rates through FY2030E. A separate procedure line multiplies volume by $20k: AMT-130 follows a centre-capacity ramp built from management's capacity comments, and Parkinson's volume follows the class-demand rule below. AMT-130 is capped by uniQure's treatable-patient estimate, then held at a steady state set as a share of HD incidence (S30). The explicit years run to FY2032E so Parkinson's volume reaches its long-run level inside the explicit years in all cases (S32). Parkinson's volume is the class of all one-time Parkinson's cell/gene therapy procedures, assumed to be 100% on ClearPoint's platform (S63): in the base and bull a three-year ramp from the FY2029E launch to class demand from the eligible-pool model in Appendix C, then class demand (S31, S44–S47, S62, S64, C40–C45); in the bear a capacity ramp to 4,500 (S22, S43). A CAL preclinical uplift is added to the biologics and drug delivery line (S37, S38). Finagle runs a 20-year horizon so growth fades gradually to its default terminal rate (S29, C29). EBITDA is gross profit (core and procedure margins) less core and procedure operating costs. The forecast was handed to Finagle with the filled inputs listed under S29, for an equity value per share. A separate Finagle rerun values a flat revenue add-on (C43). Bull and bear cases change

approval timing, ramp speed, capacity, utilisation, core totals and growth, and costs; the bull also raises the share of the Parkinson's pool treated (S59, C46), and the bear excludes patients who have had DBS from the pool (C55).